

1 AN ACT relating to judicial proceedings.

2 WHEREAS, it is the intent of this Commonwealth to promote, strengthen, and
3 encourage family life for the protection and care of children and to maintain the family
4 unit with an emphasis on the parent-child relationship; and

5 WHEREAS, the Commonwealth recognizes that parental incarceration is classified
6 as an adverse childhood experience. Multiple peer-reviewed studies demonstrate that
7 adverse childhood experiences contribute to poor mental and physical health outcomes
8 for children and increase the likelihood that a child will become involved with the
9 criminal justice system; and

10 WHEREAS, to prevent unnecessary harm to children caused by the separation from
11 a parent during incarceration, alternative sentences should be considered before a parent
12 is sentenced to a term of imprisonment; and

13 WHEREAS, this Act shall be interpreted to promote the family unit with an
14 emphasis on maintaining the parent-child relationship by providing mental health
15 treatment, substance use disorder treatment, parenting classes, educational or vocational
16 training, or other services, as appropriate, while ensuring public safety, reducing
17 recidivism, and making any victim whole through restitution;

18 NOW, THEREFORE,

19 ***Be it enacted by the General Assembly of the Commonwealth of Kentucky:***

20 ➔Section 1. KRS 533.010 is amended to read as follows:

21 (1) Any person who has been convicted of a crime and who has not been sentenced to
22 death may be sentenced to probation, probation with an alternative sentencing plan,
23 or conditional discharge as provided in this chapter.

24 (2) Before imposition of a sentence of imprisonment, the court shall consider
25 probation, probation with an alternative sentencing plan, or conditional discharge.

26 Unless the defendant is a violent felon as defined in KRS 439.3401 or a statute
27 prohibits probation, shock probation, or conditional discharge, after due

1 consideration of the defendant's risk and needs assessment, nature and
2 circumstances of the crime, and the history, character, and condition of the
3 defendant, including whether the defendant is a caretaker, probation or
4 conditional discharge shall be granted, unless the court is of the opinion that
5 imprisonment is necessary for protection of the public because:

- 6 (a) There is substantial risk that during a period of probation or conditional
7 discharge the defendant will commit another crime;
- 8 (b) The defendant is in need of correctional treatment that can be provided most
9 effectively by ~~this~~ commitment to a correctional institution; or
- 10 (c) A disposition under this chapter will unduly depreciate the seriousness of the
11 defendant's crime.

12 (3) In the event the court determines that probation is not appropriate after due
13 consideration of the defendant's risk and needs assessment, nature and
14 circumstances of the crime, and the history, character, and condition of the
15 defendant, including whether the defendant is a caretaker, probation with an
16 alternative sentencing plan shall be granted unless the court is of the opinion that
17 imprisonment is necessary for the protection of the public because:

- 18 (a) There is a likelihood that during a period of probation with an alternative
19 sentencing plan or conditional discharge the defendant will commit a Class D
20 or Class C felony or a substantial risk that the defendant will commit a Class
21 B or Class A felony;
- 22 (b) The defendant is in need of correctional treatment that can be provided most
23 effectively by commitment to a correctional institution; or
- 24 (c) A disposition under this chapter will unduly depreciate the seriousness of the
25 defendant's crime.

26 (4) The court shall not determine that there is a likelihood that the defendant will
27 commit a Class C or Class D felony based upon the defendant's risk and needs

1 assessment and the fact that:

2 (a) The defendant has never been convicted of, pled guilty to, or entered an
3 Alford plea to a felony offense;

4 (b) If convicted of, having pled guilty to, or **having** entered an Alford plea to a
5 felony offense, the defendant successfully completed probation more than ten
6 (10) years immediately prior to the date of the commission of the felony for
7 which the defendant is now being sentenced and has had no intervening
8 convictions, pleas of guilty, or Alford pleas to any criminal offense during
9 that period; or

10 (c) The defendant has been released from incarceration for the commission of a
11 felony offense more than ten (10) years immediately prior to the date of the
12 commission of the felony for which the defendant is now being sentenced and
13 has had no intervening convictions, pleas of guilty, or Alford pleas to any
14 criminal offense during that period.

15 (5) In making a determination under subsection (4) of this section, the court may
16 determine that the greater weight of the evidence indicates that there is a likelihood
17 that the defendant will commit a Class C or Class D felony.

18 (6) **In determining whether a defendant is a caretaker as described in subsections (2)**
19 **and (3) of this section, the court shall consider evidence presented to the court**
20 **that the defendant:**

21 **(a) Has consistently assumed responsibility for the housing, health, education,**
22 **safety, or support of a dependent child, family member, or any other person**
23 **identified by the court as being reliant on the defendant for support; or**

24 **(b) Is a woman who has given birth to a child while awaiting her sentencing**
25 **hearing, or who remains pregnant while awaiting her sentencing hearing.**

26 **(7)** Upon initial sentencing of a defendant or upon modification or revocation of
27 probation, when the court deems it in the best interest of the public and the

defendant, the court may order probation with the defendant to serve one (1) of the following alternative sentences:

- (a) To a halfway house for no more than twelve (12) months;
- (b) To home incarceration with or without work release for no more than twelve (12) months;
- (c) To jail for a period not to exceed twelve (12) months with or without work release, community service and other programs as required by the court;
- (d) To a residential treatment program for the abuse of alcohol or controlled substances;
- (e) To a reentry center for no more than twelve (12) months; or
- (f) To any other specified counseling program, rehabilitation or treatment program, or facility, including but not limited to one (1) of more of the following:

1. Substance use disorder treatment and prevention services;

2. Domestic violence education and prevention services;

3. Child abuse treatment and prevention services;

4. Parenting classes;

5. Anger management;

6. Vocational and educational training;

7. Targeted case management services that assist with access to transportation and affordable and safe housing;

8. Literacy and financial literacy training;

9. Individual counseling and therapy;

10. Family counseling when there is no known history of family or intimate partner violence;

11. Referral to local domestic violence resources for voluntary participation; and

1 **12. Restorative practices designed to make the participant accountable to**
2 **the victim, when there is an identified victim and when it is safe to do**
3 **so.**

4 ~~(8)~~~~(7)~~ If during the term of the alternative sentence the defendant fails to adhere to
5 and complete the conditions of the alternative sentence, the court may modify the
6 terms of the alternative sentence or may modify or revoke probation and alternative
7 sentence and commit the defendant to an institution.

8 ~~(9)~~~~(8)~~ In addition to those conditions that the court may impose, the conditions of
9 alternative sentence shall include the following and, if the court determines that the
10 defendant cannot comply with them, then they shall not be made available:

11 (a) A defendant sentenced to a halfway house shall:

- 12 1. Be working or pursuing his or her education or be enrolled in a full-time
13 treatment program;
14 2. Pay restitution during the term of probation; and
15 3. Have no contact with the victim of the defendant's crime;

16 (b) A defendant sentenced to home incarceration shall:

- 17 1. Be employed by another person or self-employed at the time of
18 sentencing to home incarceration and continue the employment
19 throughout the period of home incarceration, unless the court determines
20 that there is a compelling reason to allow home incarceration while the
21 defendant is unemployed;
22 2. Pay restitution during the term of home incarceration;
23 3. Enter a treatment program, if appropriate;
24 4. Pay all or some portion of the cost of home incarceration as determined
25 by the court;
26 5. Comply with other conditions as specified; and
27 6. Have no contact with the victim of the defendant's crime;

1 (c) A defendant sentenced to jail with community service shall:

2 1. Pay restitution during all or some part of the defendant's term of
3 probation; and

4 2. Have no contact with the victim of the defendant's crime;

5 (d) A defendant sentenced to a residential treatment program for drug and alcohol
6 abuse shall:

7 1. Undergo mandatory drug screening during term of probation;

8 2. Be subject to active, supervised probation for a term of five (5) years;

9 3. Undergo aftercare as required by the treatment program;

10 4. Pay restitution during the term of probation; and

11 5. Have no contact with the victim of the defendant's crime; or

12 (e) A defendant sentenced to a reentry center shall:

13 1. Be employed in the community or working in a vocational program at
14 the reentry center;

15 2. Be enrolled in a treatment program;

16 3. Pay restitution, fees, and fines during the term of probation; and

17 4. Comply with other conditions as specified.

18 ~~(10)~~~~(9)~~ When the court deems it in the best interest of the defendant and the public,
19 the court may order the defendant~~person~~ to work at community service related
20 projects under the terms and conditions specified in KRS 533.070. Work at
21 community service related projects shall be considered as a form of conditional
22 discharge.

23 ~~(11)~~~~(10)~~ Probation with alternative sentence shall not be available as set out in KRS
24 532.045 and 533.060, except as provided in KRS 533.030(6).

25 ~~(12)~~~~(11)~~ The court may utilize a community corrections program authorized or funded
26 under KRS Chapter 196 to provide services to any person released under this
27 section.

1 ~~(13)~~~~((12))~~ When the court deems it in the best interest of the defendant and the public,
2 the court may order the defendant to placement for probation monitoring by a
3 private agency. The private agency shall report to the court on the defendant's
4 compliance with his or her terms of probation or conditional discharge. The
5 defendant shall be responsible for any reasonable charges which the private agency
6 charges.

7 ~~(14)~~~~((13))~~ The jailer in each county incarcerating Class C or D felons may deny work
8 release privileges to any defendant for violating standards of discipline or other jail
9 regulations. The jailer shall report the action taken and the details of the violation
10 on which the action was based to the court of jurisdiction within five (5) days of the
11 violation.

12 ~~(15)~~~~((14))~~ The Department of Corrections shall, by administrative regulation, develop
13 written criteria for work release privileges granted under this section.

14 ~~(16)~~~~((15))~~ Reimbursement of incarceration costs shall be paid directly to the jailer in the
15 amount specified by written order of the court. Incarceration costs owed to the
16 Department of Corrections shall be paid through the circuit clerk.

17 ~~(17)~~~~((16))~~ The court shall enter into the record written findings of fact and conclusions
18 of law when considering implementation of any sentence under this section.

19 ➔SECTION 2. A NEW SECTION OF KRS CHAPTER 403 IS CREATED TO
20 READ AS FOLLOWS:

21 ***In any proceeding for child custody under this chapter where the court has made a***
22 ***finding that a party has committed domestic violence and abuse as defined in KRS***
23 ***403.720 against another party to the proceeding or a child of the parties:***

24 ***(1) The court shall not remove custody from or reduce the parenting time of the***
25 ***nonoffending party to whom the child is bonded or attached if the court finds that***
26 ***the:***

27 ***(a) Nonoffending party is competent and not abusive; and***

- 1 **(b) Removal would be solely for purposes of improving the relationship between**
2 **the child and the offending party; and**
3 **(2) Any order to remediate resistance of a child to contact with a violent or abusive**
4 **parent shall primarily address the behavior of that parent.**

5 ➔Section 3. KRS 403.270 is amended to read as follows:

- 6 (1) (a) As used in this chapter and KRS 405.020, unless the context requires
7 otherwise, "de facto custodian" means a person who has been shown by clear
8 and convincing evidence to have been the primary caregiver for, and financial
9 supporter of, a child who within the last two (2) years has resided with the
10 person for an aggregate period of six (6) months or more if the child is under
11 three (3) years of age and for an aggregate period of one (1) year or more if
12 the child is three (3) years of age or older or has been placed by the
13 Department for Community Based Services. Any period of time after a legal
14 proceeding has been commenced by a parent seeking to regain custody of the
15 child shall not be included in determining whether the child has resided with
16 the person for the required minimum period.
- 17 (b) A person shall not be a de facto custodian until a court determines by clear
18 and convincing evidence that the person meets the definition of de facto
19 custodian established in paragraph (a) of this subsection. Once a court
20 determines that a person meets the definition of de facto custodian, the court
21 shall give the person the same standing in custody matters that is given to
22 each parent under this section and KRS 403.280, 403.340, 403.350, 403.822,
23 and 405.020.
- 24 (2) The court shall determine custody in accordance with the best interests of the child
25 and equal consideration shall be given to each parent and to any de facto custodian.
26 Subject to KRS 403.315, there shall be a presumption, rebuttable by a
27 preponderance of evidence, that joint custody and equally shared parenting time is

1 in the best interests~~[interest]~~ of the child. If a deviation from equal parenting time is
2 warranted, the court shall construct a parenting time schedule which maximizes the
3 time each parent or de facto custodian has with the child and is consistent with
4 ensuring the child's welfare. ~~[The court shall consider all relevant factors~~
5 ~~including:]~~

6 **(3) In making a determination of custody, the court shall, prior to consideration of**
7 **any other factors enumerated in subsection (4) of this section:**

8 **(a) Consider any allegations of:**

9 **1. Domestic violence and abuse as defined in KRS 403.720; or**

10 **2. An abused or neglected child as defined in KRS 600.020;**

11 **committed by one (1) party against another party or a child of the parties;**

12 **and**

13 **(b) Make written findings on the record as to the impact, if any, of the**
14 **allegations described in paragraph (a) of this subsection on the child.**

15 **(4) After making the consideration required in subsection (3) of this section, the**
16 **court shall consider all other factors relevant to the determination of custody**
17 **including:**

18 (a) The wishes of the child's parent or parents, and any de facto custodian, as to
19 his or her custody;

20 (b) The wishes of the child as to his or her custodian, with due consideration
21 given to the influence a parent or de facto custodian may have over the child's
22 wishes;

23 (c) The interaction and interrelationship of the child with his or her parent or
24 parents, his or her siblings, and any other person who may significantly affect
25 the child's best interests;

26 (d) The motivation of the adults participating in the custody proceeding;

27 (e) The child's adjustment and continuing proximity to his or her home, school,

1 and community;

2 (f) The mental and physical health of all individuals involved;

3 (g) A finding by the court that domestic violence and abuse, as defined in KRS
4 403.720, has been committed by one (1) of the parties against a child of the
5 parties or against another party. The court shall determine the extent to which
6 the domestic violence and abuse has affected the child and the child's
7 relationship to each party, with due consideration given to efforts made by a
8 party toward the completion of any domestic violence treatment, counseling,
9 or program;

10 **(h) A finding by the court that a party has maliciously made an intentionally**
11 **false allegation of child abuse which has no basis in fact and which the**
12 **party knew had no basis in fact against another party;**

13 **(i)(h)** The extent to which the child has been cared for, nurtured, and
14 supported by any de facto custodian;

15 **(i)(i)** The intent of the parent or parents in placing the child with a de facto
16 custodian;

17 **(k)(j)** The circumstances under which the child was placed or allowed to
18 remain in the custody of a de facto custodian, including whether the parent
19 now seeking custody was previously prevented from doing so as a result of
20 domestic violence as defined in KRS 403.720 and whether the child was
21 placed with a de facto custodian to allow the parent now seeking custody to
22 seek employment, work, or attend school; and

23 **(l)(k)** The likelihood a party will allow the child frequent, meaningful, and
24 continuing contact with the other parent or de facto custodian, except that the
25 court shall not consider this likelihood if there is a finding that the other
26 parent or de facto custodian engaged in domestic violence and abuse, as
27 defined in KRS 403.720, against the party or a child and that a continuing

1 relationship with the other parent will endanger the health or safety of either
2 that party or the child.

3 ~~(5)(3)~~ The abandonment of the family residence by a custodial party shall not be
4 considered where ~~that[said]~~ party was physically harmed or was seriously
5 threatened with physical harm by his or her spouse, when ~~the[such]~~ harm or threat
6 of harm was causally related to the abandonment.

7 ~~(6)(4)~~ If the court grants custody to a de facto custodian, the de facto custodian shall
8 have legal custody under the laws of the Commonwealth.

9 ➔Section 4. KRS 403.290 is amended to read as follows:

10 (1) The court may interview the child in chambers to ascertain the child's wishes as to
11 his or her custodian and as to visitation. The court may permit counsel to be present
12 at the interview. The court shall cause a record of the interview to be made and to
13 be part of the record in the case.

14 (2) The court may seek the advice of professional personnel, whether or not employed
15 by the court on a regular basis. The advice given shall be in writing and made
16 available by the court to counsel upon request. Counsel may examine as a witness
17 any professional personnel consulted by the court.

18 **(3) (a) Paid visitation supervisors and parenting coordinators in a custody**
19 **proceeding under this chapter, whether appointed by the court or engaged**
20 **by one (1) or more parties, shall document participation training focused on**
21 **the following:**

22 **1. Early childhood, child, and adolescent development;**

23 **2. Dynamics of domestic violence;**

24 **3. Effects of domestic violence on adult and child victims;**

25 **4. Lethality and risk issues;**

26 **5. Model protocols for addressing domestic violence;**

27 **6. Available community resources and victim services; and**

1 7. For paid visitation supervisors, the role of the paid supervisor and
2 ethical principles of supervising visitation.

3 (b) The training described in paragraph (a) of this subsection shall consist of:

4 1. Sixteen (16) hours of initial training before the paid visitation
5 supervisor or parenting coordinator is eligible to provide services to a
6 party to a custody proceeding; and

7 2. Eight (8) hours annually thereafter.

8 ➔ Section 5. KRS 403.315 is amended to read as follows:

9 (1) When determining or modifying a custody order pursuant to KRS 403.270,
10 403.280, 403.340, or 403.740, the court shall consider the safety and well-being of
11 the parties and of the children. If a domestic violence order is being or has been
12 entered against a party by another party or on behalf of a child at issue in the
13 custody hearing, the presumption that joint custody and equally shared parenting
14 time is in the best interests~~[interest]~~ of the child shall not apply as to the party
15 against whom the domestic violence order is being or has been entered. The court
16 shall weigh all factors set out in Section 3 of this Act~~[KRS 403.270(2)]~~ in
17 determining the best interests~~[interest]~~ of the child.

18 (2) There shall be a presumption, rebuttable by a preponderance of the evidence, that
19 joint custody and equally shared parenting time is not in the best interests of the
20 child if the court finds that a party has committed two (2) or more acts of
21 domestic violence and abuse as defined in KRS 403.720 against another party.

22 (3) (a) The court shall not make an award of custody or grant unsupervised
23 visitation with the child to a party who is found to have committed two (2) or
24 more acts of domestic violence or abuse as defined in KRS 403.720 against
25 another party unless that party has:

26 1. Completed:

27 a. A batterer's intervention program provided by a certified

1 provider pursuant to 920 KAR 5:020;

2 b. Parenting classes; and

3 c. Substance use and mental health assessments and any
4 recommended corresponding treatments; and

5 2. Participated in supervised visitation.

6 (b) Following a party's completion of the requirements in paragraph (a) of this
7 subsection, the court shall conduct an evidentiary hearing before ordering
8 unsupervised visitation to that party and a second or subsequent hearing
9 before awarding custodial rights to that party.

10 ➔Section 6. KRS 403.727 is amended to read as follows:

11 (1) Any petition filed under KRS 403.725 on behalf of a minor who is alleged to be a
12 victim of domestic violence and abuse, or in which a minor is named as a
13 respondent or petitioner, shall comply with the requirements in that section and
14 shall:

15 (a) Proceed in accordance with the procedural safeguards under KRS 610.070;
16 and

17 (b) Conform to the confidentiality provisions under KRS 610.340.

18 (2) (a) If the court orders an evidentiary hearing under KRS 403.730(1)(a), a
19 guardian ad litem shall be appointed for any unrepresented minor who is a
20 respondent to the action or a petitioner who is an alleged victim of domestic
21 violence and abuse.

22 (b) The guardian ad litem shall be paid a fee fixed by the court not to exceed five
23 hundred dollars (\$500), which shall be paid by the Finance and
24 Administration Cabinet.

25 (3) A victim advocate as defined in KRS 421.570:

26 (a) Shall be permitted to attend the evidentiary hearing to offer advocacy
27 services pursuant to KRS 421.575 to the minor petitioner or the adult who

1 has filed the petition on behalf of the minor; and

2 (b) Shall not be excluded from the evidentiary hearing unless the:

3 1. Minor petitioner or adult who has filed the petition on behalf of the
4 minor declines the services offered; or

5 2. Court, upon motion and for good cause shown, determines that the
6 exclusion is in the best interests of the minor petitioner.

7 ~~(4)~~~~(3)~~ Violation of the terms or conditions of an order of protection issued under
8 KRS 403.740 after the person has been served or given notice of the order shall
9 constitute contempt of court and may constitute a criminal offense pursuant to KRS
10 403.763 if the offender is an adult or a public offense under KRS 600.020~~(51)~~ if
11 the offender is a juvenile. Once a juvenile action or contempt proceeding has been
12 initiated, the other shall not be undertaken regardless of the outcome of the original
13 proceeding.

14 ~~(5)~~~~(4)~~ Nothing in subsection ~~(4)~~~~(3)~~ of this section shall preclude the
15 Commonwealth from proceeding, or the petitioner from pursuing charges, against
16 the minor respondent for offenses other than a violation of an order of protection.
17 Proceedings against a minor respondent for offenses other than a violation of an
18 order of protection shall proceed:

19 (a) In the juvenile session of District Court; and

20 (b) In accordance with the procedural and statutory provisions established for the
21 juvenile session of District Court.

22 ➔Section 7. KRS 456.035 is amended to read as follows:

23 (1) Any petition filed under KRS 456.030 on behalf of a minor who is an alleged
24 victim of dating violence and abuse, sexual assault, or stalking, or in which a minor
25 is named as a respondent or petitioner, shall comply with the requirements in that
26 section and shall:

27 (a) Proceed in accordance with the procedural safeguards under KRS 610.070;

1 and

2 (b) Conform to the confidentiality provisions under KRS 610.340.

3 (2) (a) If the court orders an evidentiary hearing under KRS 456.040(1)(a), a
4 guardian ad litem shall be appointed for any unrepresented minor who is a
5 respondent to the action or a petitioner who is an alleged victim of dating
6 violence and abuse, sexual assault, or stalking.

7 (b) The guardian ad litem shall be paid a fee fixed by the court not to exceed five
8 hundred dollars (\$500), which shall be paid by the Finance and
9 Administration Cabinet.

10 (3) A victim advocate as defined in KRS 421.570:

11 (a) Shall be permitted to attend the evidentiary hearing to offer advocacy
12 services pursuant to KRS 421.575 to the minor petitioner or the adult who
13 has filed the petition on behalf of the minor; and

14 (b) Shall not be excluded from the evidentiary hearing unless the:

15 1. Minor petitioner or adult who has filed the petition on behalf of the
16 minor declines the services offered; or

17 2. Court, upon motion and for good cause shown, determines that the
18 exclusion is in the best interests of the minor petitioner.

19 ~~(4)(3)~~ Violation of the terms or conditions of an order of protection issued under
20 KRS 456.060 after the person has been served or given notice of the order shall
21 constitute contempt of court and may constitute a criminal offense pursuant to KRS
22 456.180 if the offender is an adult or a public offense under KRS 600.020~~(51)~~ if
23 the offender is a juvenile. Once a juvenile action or contempt proceeding has been
24 initiated, the other shall not be undertaken regardless of the outcome of the original
25 proceeding.

26 ~~(5)(4)~~ Nothing in subsection ~~(4)(3)~~ of this section shall preclude the
27 Commonwealth from proceeding, or the petitioner from pursuing charges, against

1 the minor respondent for offenses other than a violation of an order of protection.
2 Proceedings against a minor respondent for offenses other than a violation of an
3 order of protection shall proceed:

4 (a) In the juvenile session of District Court; and

5 (b) In accordance with the procedural and statutory provisions established for the
6 juvenile session of District Court.

7 ➔Section 8. KRS 202A.011 is amended to read as follows:

8 As used in this chapter, unless the context otherwise requires:

9 (1) "Authorized staff physician" means a physician who is a bona fide member of the
10 hospital's medical staff;

11 (2) **"Benefit from treatment" means the desired outcomes of treatment in a**
12 **psychiatric hospital for an individual with a mental illness, including but not**
13 **limited to:**

14 **(a) Symptom management and increased stability;**

15 **(b) A lessening of irrational thoughts and behaviors;**

16 **(c) Reduced risk of harm; or**

17 **(d) Acquisition of skills for self-care and for interacting and living in the**
18 **community;**

19 **(3)** "Cabinet" means the ~~[Kentucky]~~Cabinet for Health and Family Services;

20 **(4)**~~[(3)]~~ "Contract mental health evaluator" means a qualified mental health
21 professional who is employed by or under contract with a community mental health
22 center, crisis stabilization unit, mental institution, or any other facility designated by
23 the secretary to provide mental health evaluations to determine whether an
24 individual meets the criteria for involuntary hospitalization;

25 **(5)**~~[(4)]~~ "Danger" or "threat of danger to self, family, or others" means, **as a result of**
26 **mental illness, a person:**

27 **(a) Presents a** substantial **risk of serious** physical harm ~~to~~**for threat of substantial**

1 ~~physical harm upon~~ self, family, or others as evidenced by recent behavior,
2 threats, or conduct demonstrating such risk;~~;~~

3 **(b) Has attempted or threatened suicide or has expressed suicidal ideations and**
4 **there is a reasonable probability of serious self-harm unless prompt and**
5 **adequate treatment is provided;**

6 **(c) Is unable without supervision or assistance to provide for**~~including actions~~
7 ~~which deprive self, family, or others of the~~ basic **personal needs,**~~means of~~
8 ~~survival~~ including provision for reasonable shelter, food, ~~for~~ clothing, **or**
9 **medical care, so that there exists a substantial likelihood of death, serious**
10 **physical injury, or serious physical debilitation. For purposes of this**
11 **paragraph, a person shall be deemed unable to provide for basic personal**
12 **needs even if a guardian, family member, or friend is willing and able to**
13 **provide assistance; or**

14 **(d) Is experiencing psychiatric deterioration, demonstrated by a substantial**
15 **decline in functioning from the person's baseline, such that the person's**
16 **judgment, insight, or ability to recognize the need for treatment is impaired,**
17 **and without intervention, the person's condition will predictably result in:**
18 **1. A substantial risk of serious physical harm to self, family, or others;**
19 **2. Serious physical debilitation or self-neglect; or**
20 **3. Further loss of ability to engage in safe or necessary self-care or to**
21 **voluntarily seek needed treatment;**

22 **(6)**~~(5)~~ "Forensic psychiatric facility" means a mental institution or facility, or part
23 thereof, designated by the secretary for the purpose and function of providing
24 inpatient evaluation, care, and treatment for ~~mentally ill persons or~~ individuals
25 with an intellectual disability **or mental illness**, who have been charged with or
26 convicted of a felony;

27 **(7)**~~(6)~~ "Hospital" means:

1 (a) A state mental hospital or institution or other licensed public or private
2 hospital, institution, health-care facility, or part thereof, approved by the
3 cabinet~~[Kentucky Cabinet for Health and Family Services]~~ as equipped to
4 provide full-time residential care and treatment for ~~[mentally ill persons or~~
5 ~~]individuals with an intellectual disability~~ **or mental illness**; or

6 (b) A hospital, institution, or health-care facility of the government of the United
7 States equipped to provide residential care and treatment for ~~[mentally ill~~
8 ~~persons or]individuals with an intellectual disability~~ **or mental illness**;

9 **(8) "Individual with a mental illness" means a person with substantially impaired**
10 **capacity to use self-control, judgment, or discretion in the conduct of the person's**
11 **affairs and social relations, associated with maladaptive behavior or recognized**
12 **emotional symptoms where impaired capacity, maladaptive behavior, or**
13 **emotional symptoms can be related to physiological, psychological, or social**
14 **factors;**

15 **(9)**~~[(7)]~~ "Judge" means any judge or justice of the Court of Justice or a trial
16 commissioner of the District Court acting under authority of SCR 5.030;

17 **(10)**~~[(8)]~~ "Least restrictive alternative mode of treatment" means that treatment which
18 will give an~~[a mentally ill]~~ individual **with a mental illness** a realistic opportunity
19 to improve the individual's level of functioning, consistent with accepted
20 professional practice in the least confining setting available;{

21 ~~(9) "Mentally ill person" means a person with substantially impaired capacity to use~~
22 ~~self-control, judgment, or discretion in the conduct of the person's affairs and social~~
23 ~~relations, associated with maladaptive behavior or recognized emotional symptoms~~
24 ~~where impaired capacity, maladaptive behavior, or emotional symptoms can be~~
25 ~~related to physiological, psychological, or social factors;]~~

26 **(11)**~~[(10)]~~ "Patient" means a person under observation, care, or treatment in a hospital
27 pursuant to the provisions of this chapter;

1 ~~(12)~~~~(11)~~ "Petitioner" means a person who institutes a proceeding under this chapter;

2 ~~(13)~~~~(12)~~ "Psychiatric facility" means a crisis stabilization unit or any facility licensed
3 by the cabinet and which provides inpatient, outpatient, psychosocial rehabilitation,
4 emergency, and consultation and education services for the diagnosis and treatment
5 of persons who have a mental illness;

6 ~~(14)~~~~(13)~~ "Qualified mental health professional" means:

- 7 (a) A physician licensed under the laws of Kentucky to practice medicine or
8 osteopathy, or a medical officer of the government of the United States while
9 engaged in the performance of official duties;
- 10 (b) A psychiatrist licensed under the laws of Kentucky to practice medicine or
11 osteopathy, or a medical officer of the government of the United States while
12 engaged in the practice of official duties, who is certified or eligible to apply
13 for certification by the American Board of Psychiatry and Neurology, Inc.;
- 14 (c) A psychologist with the health service provider designation, a psychological
15 practitioner, a certified psychologist, or a psychological associate, licensed
16 under the provisions of KRS Chapter 319;
- 17 (d) A licensed registered nurse with a master's degree in psychiatric nursing from
18 an accredited institution and two (2) years of clinical experience with
19 individuals with a mental illness~~mentally ill persons~~, or a licensed
20 registered nurse, with a bachelor's degree in nursing from an accredited
21 institution, who is certified as a psychiatric and mental health nurse by the
22 American Nurses Association and who has three (3) years of inpatient or
23 outpatient clinical experience in psychiatric nursing and is currently employed
24 by a hospital or forensic psychiatric facility licensed by the Commonwealth or
25 a psychiatric unit of a general hospital or a private agency or company
26 engaged in the provision of mental health services or a regional community
27 program for mental health and individuals with an intellectual disability;

- 1 (e) A licensed clinical social worker licensed under the provisions of KRS
2 335.100, or a certified social worker licensed under the provisions of KRS
3 335.080 with three (3) years of inpatient or outpatient clinical experience in
4 psychiatric social work and currently employed by a hospital or forensic
5 psychiatric facility licensed by the Commonwealth or a psychiatric unit of a
6 general hospital or a private agency or company engaged in the provision of
7 mental health services or a regional community program for mental health and
8 individuals with an intellectual disability;
- 9 (f) A marriage and family therapist licensed under the provisions of KRS
10 335.300 to 335.399 with three (3) years of inpatient or outpatient clinical
11 experience in psychiatric mental health practice and currently employed by a
12 hospital or forensic facility licensed by the Commonwealth, a psychiatric unit
13 of a general hospital, a private agency or company engaged in providing
14 mental health services, or a regional community program for mental health
15 and individuals with an intellectual disability;
- 16 (g) A professional counselor credentialed under the provisions of KRS Chapter
17 335.500 to 335.599 with three (3) years of inpatient or outpatient clinical
18 experience in psychiatric mental health practice and currently employed by a
19 hospital or forensic facility licensed by the Commonwealth, a psychiatric unit
20 of a general hospital, a private agency or company engaged in providing
21 mental health services, or a regional community program for mental health
22 and individuals with an intellectual disability; or
- 23 (h) A physician assistant licensed under KRS 311.840 to 311.862, who meets one
24 (1) of the following requirements:
- 25 1. Provides documentation that he or she has completed a psychiatric
26 residency program for physician assistants;
 - 27 2. Has completed at least one thousand (1,000) hours of clinical experience

1 under a supervising physician, as defined by KRS 311.840, who is a
2 psychiatrist and is certified or eligible for certification by the American
3 Board of Psychiatry and Neurology, Inc.;

4 3. Holds a master's degree from a physician assistant program accredited
5 by the Accreditation Review Commission on Education for the
6 Physician Assistant or its predecessor or successor agencies, is
7 practicing under a supervising physician as defined by KRS 311.840,
8 and:

9 a. Has two (2) years of clinical experience in the assessment,
10 evaluation, and treatment of mental disorders; or

11 b. Has been employed by a hospital or forensic psychiatric facility
12 licensed by the Commonwealth or a psychiatric unit of a general
13 hospital or a private agency or company engaged in the provision
14 of mental health services or a regional community program for
15 mental health and individuals with an intellectual disability for at
16 least two (2) years; or

17 4. Holds a bachelor's degree, possesses a current physician assistant
18 certificate issued by the board prior to July 15, 2002, is practicing under
19 a supervising physician as defined by KRS 311.840, and:

20 a. Has three (3) years of clinical experience in the assessment,
21 evaluation, and treatment of mental disorders; or

22 b. Has been employed by a hospital or forensic psychiatric facility
23 licensed by the Commonwealth or a psychiatric unit of a general
24 hospital or a private agency or company engaged in the provision
25 of mental health services or a regional community program for
26 mental health and individuals with an intellectual disability for at
27 least three (3) years;

1 ~~(15)~~~~((14))~~ "Residence" means legal residence as determined by applicable principles
2 governing conflicts of law;

3 ~~(16)~~~~((15))~~ "Respondent" means a person alleged in a hearing under this chapter to be ~~a~~
4 ~~mentally ill person or~~ an individual with an intellectual disability or mental
5 illness~~; and~~

6 ~~(17)~~~~((16))~~ "Secretary" means the secretary of the Cabinet for Health and Family
7 Services~~[-]; and~~

8 **(18) "Serious mental illness":**

9 **(a) Means a diagnosable mental, behavioral, or emotional disorder that causes**
10 **significant functional impairment that substantially interferes with or limits**
11 **major life activities, including but not limited to:**

12 **1. Severe cognitive difficulties, including disorganized thinking,**
13 **delusions, hallucinations, or memory impairment; or**

14 **2. Extreme mood fluctuations, apathy, or lack of motivation; and**

15 **(b) Includes but is not limited to the following disorders:**

16 **1. Schizophrenia spectrum and other psychotic disorders;**

17 **2. Bipolar and related disorders; and**

18 **3. Major depressive disorders that require treatment.**

19 ➔Section 9. KRS 202A.028 is amended to read as follows:

20 (1) **Prior to completion of an examination by a qualified mental health professional**
21 **under this section, the professional shall make a good-faith attempt to contact the**
22 **petitioner to obtain any additional relevant information necessary to the petition.**

23 (2) (a) **Prior to the release of a person subject to an examination under this**
24 **section, the county attorney may make an ex parte motion for a certification**
25 **review hearing. The court shall review the ex parte motion upon its receipt.**

26 (b) **If the review indicates that the person presents an imminent threat of**
27 **danger to self, family, or others, the court shall order the qualified mental**

1 health professional to immediately notify the court if the professional has
2 certified that the person:

3 1. Is an individual with a mental illness;

4 2. Presents a danger or threat of danger to self, family, or others as a
5 result of the mental illness; and

6 3. Does not meet all the criteria for involuntary hospitalization under
7 Section 34 of this Act.

8 (c) The court shall review the certification under paragraph (b) of this
9 subsection and consider if the person has been the subject of proceedings
10 under this chapter, prior to the current proceeding, and is exhibiting an
11 escalation of dangerous behavior. After review, the court may:

12 1. Order the person to be hospitalized in a place designated by the
13 cabinet until the certification review hearing is held within forty-eight
14 (48) hours, excluding weekends and holidays;

15 2. Release the person, subject to reasonable conditions, and set the
16 certification review hearing to be held within forty-eight (48) hours,
17 excluding weekends and holidays; or

18 3. Deny the county attorney's motion for a certification review and
19 authorize the release of the person.

20 (d) 1. At the certification review hearing, the person shall be appointed
21 counsel who may present evidence and cross examine witnesses on the
22 person's behalf.

23 2. The qualified mental health professional that performed the
24 examination shall testify about the certification under paragraph (b)
25 of this subsection. This testimony may be given remotely.

26 3. Notwithstanding subparagraph 2. of this paragraph, the court may
27 accept the certification under paragraph (b) of this subsection in lieu

1 of testimony upon agreement of all parties and stipulation of the
2 person that he or she meets the criteria for involuntary hospitalization
3 under Section 34 of this Act, provided that outpatient treatment is
4 determined to be the least restrictive mode of treatment.

5 (e) Upon conclusion of the certification review hearing, if the court finds by
6 clear and convincing evidence that the person is an individual with a mental
7 illness who presents a danger or threat of danger to self, family, or others as
8 a result of the mental illness, who may reasonably benefit from court-
9 ordered outpatient treatment or release with other reasonable conditions,
10 and for whom court-ordered outpatient treatment or release with other
11 reasonable conditions is the least restrictive alternative mode of treatment
12 available, the court may order the person to:

- 13 1. Receive community-based outpatient treatment that shall not exceed
14 three hundred sixty (360) days and comply with any other reasonable
15 conditions necessary to ensure compliance; or
16 2. Be released with any reasonable conditions necessary to ensure the
17 safety of self, family, or others and avoid readmittance to a hospital
18 setting.

19 (3) (a) Failure to comply with an order for community-based outpatient treatment
20 or release with conditions under subsection (2)(e) of this section shall not be
21 grounds to find the person in contempt of court but shall be prima facie
22 evidence that:

- 23 1. The person can benefit from inpatient hospitalization; and
24 2. Inpatient hospitalization is the least restrictive mode of treatment.

25 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
26 months of an order for community-based outpatient treatment or release
27 with conditions under subsection (2)(e) of this section, the clerk of the court

1 shall provide the prior order to:

2 1. The court with the petition for involuntary hospitalization; and

3 2. The qualified mental health professional along with the court's order
4 for examination.

5 (c) The qualified mental health professional who conducts the examination as
6 described under paragraph (b)2. of this subsection shall independently
7 evaluate the person, considering the person's prior order for community-
8 based outpatient treatment or release with conditions, and certify whether
9 he or she meets the criteria for involuntary hospitalization under Section 34
10 of this Act.

11 (d) A person shall not be involuntarily hospitalized based solely on his or her
12 failure to comply with a prior order for community-based outpatient
13 treatment or release with conditions.

14 **(4)** Following an examination by a qualified mental health professional and a
15 certification by that professional that the person meets the criteria for involuntary
16 hospitalization, a judge may order the person hospitalized for a period not to exceed
17 seventy-two (72) hours, excluding weekends and holidays. For the purposes of this
18 section, the qualified mental health professional shall be:

19 (a) A staff member of a regional community program for mental health or
20 individuals with an intellectual disability;

21 (b) An individual qualified and licensed to perform the examination through the
22 use of telehealth services; or

23 (c) The psychiatrist ordered, subject to the court's discretion, to perform the
24 required examination.

25 ~~**(5)**~~ ~~**(2)**~~ Any person who has been admitted to a hospital under subsection (1) of this
26 section shall be released from the hospital within seventy-two (72) hours, excluding
27 weekends and holidays, unless further held under the applicable provisions of this

1 chapter.

2 ~~(6)~~~~(3)~~ **(a)** Any person admitted to a hospital under subsection (1) of this section or
3 transferred to a hospital while ordered hospitalized under subsection (1) of
4 this section shall be transported from the person's home county by the sheriff
5 of that county or other peace officer as ordered by the court.

6 **(b)** The sheriff or other peace officer may, upon agreement of a person authorized
7 by the peace officer, authorize the cabinet, a private agency on contract with
8 the cabinet, or an ambulance service designated by the cabinet to transport the
9 person to the hospital.

10 **(c)** The transportation costs of the sheriff, other peace officer, ambulance service,
11 or other private agency on contract with the cabinet shall be paid by the
12 cabinet in accordance with an administrative regulation promulgated by the
13 cabinet **in accordance with**~~[, pursuant to]~~ KRS Chapter 13A.

14 ~~(7)~~~~(4)~~ **(a)** Any person released from the hospital under subsection (2) of this
15 section shall be transported to the person's county of discharge by a sheriff or
16 other peace officer, by an ambulance service designated by the cabinet, or by
17 other appropriate means of transportation which is consistent with the
18 treatment plan of that person.

19 **(b)** The transportation cost of transporting the patient to the patient's county of
20 discharge when performed by a peace officer, ambulance service, or other
21 private agency on contract with the cabinet shall be paid by the cabinet in
22 accordance with an administrative regulation **promulgated**~~[issued]~~ by the
23 cabinet **in accordance with**~~[, pursuant to]~~ KRS Chapter 13A.

24 ~~(8)~~~~(5)~~ ~~A~~~~Not~~ person who has been held under subsection (1) of this section shall **not**
25 be held in jail pending evaluation and transportation to the hospital.

26 **(9) A court order under subsection (4) of this section shall expire after thirty (30)**
27 **days if not served upon the person subject to the order.**

1 ➔ SECTION 10. A NEW SECTION OF KRS CHAPTER 202A IS CREATED
2 TO READ AS FOLLOWS:

3 (1) (a) Upon motion of the county attorney, the hospital shall give notice to the
4 court and the county attorney if the hospital plans to discharge the
5 respondent following the certification by a qualified mental health
6 professional and before the preliminary hearing. Prior to discharge, the
7 county attorney may make an ex parte motion for a certification review
8 hearing. The court shall review the ex parte motion upon its receipt.

9 (b) If the review indicates that the respondent presents an imminent threat of
10 danger to self, family, or others, the court shall order the qualified mental
11 health professional to immediately notify the court if the professional has
12 certified that the respondent:

13 1. Is an individual with a mental illness;

14 2. Presents a danger or threat of danger to self, family, or others as a
15 result of the mental illness; and

16 3. Does not meet all the criteria for involuntary hospitalization under
17 Section 34 of this Act.

18 (c) The court shall review the certification under paragraph (b) of this
19 subsection and consider if the respondent has been the subject of
20 proceedings under this chapter, prior to the current proceeding, and is
21 exhibiting an escalation of dangerous behavior. After review, the court
22 may:

23 1. Order the respondent to be hospitalized in a place designated by the
24 cabinet until the certification review hearing is held within forty-eight
25 (48) hours, excluding weekends and holidays;

26 2. Release the respondent, subject to reasonable conditions, and set the
27 certification review hearing to be held within forty-eight (48) hours; or

1 3. Deny the county attorney's motion for a certification review and
2 authorize the release of the person.

3 (d) 1. At the certification review hearing, the respondent shall be appointed
4 counsel who may present evidence and cross examine witnesses on the
5 respondent's behalf.

6 2. The qualified mental health professional that performed the
7 examination shall testify about the certification under paragraph (b)
8 of this subsection. This testimony may be given remotely.

9 3. Notwithstanding subparagraph 2. of this paragraph, the court may
10 accept the certification under paragraph (b) of this subsection in lieu
11 of testimony upon agreement of all parties and stipulation of the
12 respondent that he or she meets the criteria for involuntary
13 hospitalization under Section 34 of this Act, provided that outpatient
14 treatment is determined to be the least restrictive mode of treatment.

15 (e) Upon conclusion of the certification review hearing, if the court finds by
16 clear and convincing evidence that the respondent is an individual with a
17 mental illness who presents a danger or threat of danger to self, family, or
18 others as a result of the mental illness, who may reasonably benefit from
19 court-ordered outpatient treatment or release with reasonable conditions,
20 and for whom court-ordered outpatient treatment or release with reasonable
21 conditions is the least restrictive alternative mode of treatment available, the
22 court may order the respondent to:

23 1. Receive community-based outpatient treatment that shall not exceed
24 three hundred sixty (360) days and comply with any other reasonable
25 conditions necessary to ensure compliance; or

26 2. Be released with any reasonable conditions necessary to ensure the
27 safety of self, family, or others and avoid readmittance to a hospital

1 setting.

2 (2) (a) Failure to comply with an order for community-based outpatient treatment
3 or release with conditions under subsection (1)(e) of this section shall not be
4 grounds to find the respondent in contempt of court but shall be prima facie
5 evidence that:

6 1. The respondent can benefit from inpatient hospitalization; and

7 2. Inpatient hospitalization is the least restrictive mode of treatment.

8 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
9 months of an order for community-based outpatient treatment or release
10 with conditions under subsection (1)(e) of this section, the clerk of the court
11 shall provide the prior order to:

12 1. The court with the petition for involuntary hospitalization; and

13 2. The qualified mental health professional along with the court's order
14 for examination.

15 (c) The qualified mental health professional who conducts the examination as
16 described under paragraph (b)2. of this subsection shall independently
17 evaluate the respondent, considering the respondent's prior order for
18 community-based outpatient treatment or release with conditions, and
19 certify whether he or she meets the criteria for involuntary hospitalization
20 under Section 34 of this Act.

21 (d) A respondent shall not be involuntarily hospitalized based solely on his or
22 her failure to comply with a prior order for community-based outpatient
23 treatment or release with conditions.

24 ➔ Section 11. KRS 202A.051 is amended to read as follows:

25 (1) Proceedings for up to sixty (60) days or up to three hundred sixty (360) days of
26 involuntary hospitalization of an individual shall be initiated by the filing of a
27 verified petition in District Court.

- 1 (2) The petition and all subsequent court documents shall be entitled: "In the interest of
2 (name of respondent)."
- 3 (3) The petition shall be filed by a qualified mental health professional, peace officer,
4 county attorney, Commonwealth's attorney, spouse, relative, friend, or guardian of
5 the individual concerning whom the petition is filed, or any **responsible adult**~~[other~~
6 ~~interested person]~~.
- 7 (4) The petition shall set forth:
- 8 (a) Petitioner's relationship to the respondent;
- 9 (b) Respondent's name, residence, and current location, if known;
- 10 (c) The name and residence of respondent's parents, if living and if known, or
11 respondent's legal guardian, if any and if known;
- 12 (d) The name and residence of respondent's husband or wife, if any and if known;
- 13 (e) The name and residence of the person having custody of the respondent, if
14 any, or if no such person is known, the name and residence of a near relative
15 or that the person is unknown;
- 16 (f) Petitioner's belief, including the factual basis therefor, that the respondent is
17 **an individual with a mental illness**~~[mentally ill]~~ and presents a danger or
18 threat of danger to self, family or others if not restrained;~~[and]~~
- 19 (g) If the petition seeks a three hundred sixty (360) day involuntary
20 hospitalization of the respondent, the petition shall further set forth that the
21 respondent has been hospitalized in a hospital or a forensic psychiatric facility
22 for a period of thirty (30) days under the provisions of this chapter or KRS
23 Chapter 504 within the preceding six (6) months; **and**
- 24 **(h) Upon request of the county attorney, that the county attorney receives notice**
25 **of any discharge by a hospital prior to the preliminary hearing under**
26 **subsection (10) of this section.**
- 27 (5) **If the petition seeks a sixty (60) day involuntary hospitalization of the respondent,**

the county attorney may motion the court to amend the petition to seek up to a three hundred sixty (360) day involuntary hospitalization of the individual if the respondent has been hospitalized in a hospital or a forensic psychiatric facility for a period of thirty (30) days under the provisions of this chapter or KRS Chapter 504 within the preceding six (6) months.

(6) Upon receipt of the petition, the court shall examine the petitioner under oath as to the contents of the petition. If the petitioner is a qualified mental health professional, the court may dispense with the examination.

~~**(7)**~~~~**(6)**~~ If after reviewing the allegations contained in the petition and examining the petitioner under oath, it appears to the court that there is probable cause to believe the respondent should be involuntarily hospitalized, the court shall, unless either the court or one (1) of the parties objects, implement the procedures provided in KRS 202A.028 and order the individual to be examined without unnecessary delay by a qualified mental health professional. If the person is not being hospitalized~~held~~ under the provisions of this chapter, the court may order that the sheriff of the county or other peace officer transport the person to a hospital or psychiatric facility designated by the cabinet for the purpose of the evaluation. The sheriff or other peace officer may, upon agreement of a person authorized by the peace officer, authorize the cabinet, a private agency on contract with the cabinet, or an ambulance service designated by the cabinet to transport the person to a hospital or psychiatric facility. Following that, the procedures as set forth in KRS 202A.028 shall be carried out. Otherwise, the court shall:

- (a) Set a date for a preliminary hearing within six (6) days from the date of hospitalization of~~holding~~ the person under the provisions of this section, ~~section (excluding holidays and weekends,)~~ to determine if there is probable cause to believe the person should be involuntarily hospitalized;
- (b) Notify the respondent, the legal guardian, if any, and if known, and the

1 spouse, parents, or nearest relative or friend of the respondent concerning the
2 allegations and contents of the petition and the date and purpose of the
3 preliminary hearing; and the name, address, and telephone number of the
4 attorney appointed to represent the respondent; and

5 (c) Cause the respondent to be examined without unnecessary delay by two (2)
6 qualified mental health professionals, at least one (1) of whom is a physician.
7 The qualified mental health professionals shall certify within twenty-four (24)
8 hours, ~~{(excluding weekends and holidays,)}~~ their findings.

9 ~~(8)~~~~(7)~~ (a) If the respondent is being presently ~~hospitalized~~~~{held}~~ under the
10 provisions of this chapter, the court may order further
11 ~~hospitalization~~~~{holding}~~ of the respondent to accomplish the examination
12 ordered by the court.

13 (b) If the respondent is not being presently ~~hospitalized~~~~{held}~~ under the
14 provisions of this chapter, the court may order that the sheriff of the county or
15 a peace officer transport the respondent to a hospital or a psychiatric facility
16 designated by the cabinet so that the respondent shall be examined without
17 unnecessary delay by two (2) qualified mental health professionals, at least
18 one (1) of whom is a physician. The sheriff or other peace officer may
19 authorize, upon agreement of a person authorized by the peace officer, the
20 cabinet, a private agency on contract with the cabinet, or an ambulance
21 service designated by the cabinet to transport the person to a hospital or
22 psychiatric facility.

23 ~~(9)~~~~(8)~~ (a) When the court is authorized to issue an order that the respondent be
24 transported to a hospital or psychiatric facility, the court may, in its discretion,
25 issue a summons. A summons so issued shall be directed to the respondent,
26 shall command the respondent to appear at a time and place ~~{therein}~~ specified
27 in the summons where the respondent shall be ~~{there}~~ examined by two (2)

1 qualified mental health professionals, at least one (1) of whom is a physician,
2 and shall command the respondent's appearance at the preliminary hearing.

3 **(b)** If a respondent who has been summoned fails to appear for such examination
4 or at the preliminary hearing, the court may order that the sheriff of the county
5 or a peace officer transport the respondent to a hospital or psychiatric facility
6 designated by the cabinet for the purpose of an evaluation.

7 **(10)**~~**(9)**~~ If upon completion of the preliminary hearing, the court finds:

8 **(a)** There is probable cause to believe the respondent should be involuntarily
9 hospitalized;~~;~~

10 **1.** The court shall order a final hearing within twenty-one (21) days from
11 the date of **hospitalization of**~~**holding**~~ the respondent under ~~the~~
12 ~~provisions of~~ this section to determine if the respondent should
13 **continue to** be involuntarily hospitalized; ~~and~~

14 **2. a. The county attorney may motion the court at the conclusion of**
15 **the preliminary hearing to require the hospital to provide a copy**
16 **of the respondent's discharge plan to the county attorney prior to**
17 **discharge. If the hospital plans to discharge the respondent prior**
18 **to the final hearing under subsection (11) of this section, the**
19 **court shall require the hospital to provide a copy of the**
20 **respondent's discharge plan no later than seventy-two (72) hours**
21 **prior to the respondent's discharge.**

22 **b. If the county attorney believes that the discharge plan does not**
23 **give the respondent a realistic opportunity to avoid imminent**
24 **readmittance into an inpatient psychiatric hospital for treatment**
25 **and is insufficient without court-ordered outpatient treatment or**
26 **reasonable conditions, the county attorney shall make an ex**
27 **parte motion for a discharge review hearing. The court shall**

1 conduct the discharge review hearing at the earliest practicable
2 time, consistent with due process and the availability of counsel,
3 and in no event later than seven (7) days after the motion is
4 made, except with the respondent's consent or upon the court's
5 finding that extraordinary circumstances exist and that delay is
6 indispensable to protect the respondent and the community.

7 c. Upon completion of the discharge review hearing described
8 under subdivision b. of this subparagraph, if the court finds that
9 the respondent is an individual with a mental illness who
10 presents a danger or threat of danger to self, family, or others as
11 a result of the mental illness, who may reasonably benefit from
12 court-ordered outpatient treatment or release with other
13 reasonable conditions, and for whom court-ordered outpatient
14 treatment or release with other reasonable conditions is the least
15 restrictive alternative mode of treatment available, the court shall
16 order:

17 i. For a respondent who meets the criteria for court-ordered
18 assisted outpatient treatment set forth in Section 20 of this
19 Act, that a treatment plan be developed in accordance with
20 KRS 202A.0817 within forty-eight (48) hours and the
21 respondent to receive court-ordered assisted outpatient
22 treatment under KRS 202A.0811 to 202A.0831;

23 ii. For any other respondent who may benefit from outpatient
24 treatment, a qualified mental health professional to develop
25 a treatment plan within forty-eight (48) hours, the
26 respondent to receive community-based outpatient
27 treatment that shall not exceed three hundred sixty (360)

- 1 days, and the respondent to comply with any other
2 reasonable conditions necessary to ensure compliance; or
3 iii. The respondent to be released with any reasonable
4 conditions necessary to ensure the safety of self, family, or
5 others and avoid readmittance into a hospital setting;
6 (b) There is no probable cause to believe the respondent should be involuntarily
7 hospitalized but that the respondent is an individual with a mental illness
8 who presents a danger or threat of danger to self, family, or others as a
9 result of the mental illness, who may reasonably benefit from court-ordered
10 outpatient treatment or release with other reasonable conditions, and for
11 whom court-ordered outpatient treatment or release with other reasonable
12 conditions is the least restrictive alternative mode of treatment available, the
13 court shall order:
14 1. For a respondent who meets the criteria for court-ordered assisted
15 outpatient treatment set forth in Section 20 of this Act:
16 a. That a treatment plan be developed in accordance with KRS
17 202A.0817 within forty-eight (48) hours; and
18 b. The respondent to receive court-ordered assisted outpatient
19 treatment under KRS 202A.0811 to 202A.0831;
20 2. For any other respondent who may benefit from outpatient treatment:
21 a. A qualified mental health professional to develop a treatment
22 plan within forty-eight (48) hours;
23 b. The respondent to receive community-based outpatient treatment
24 that shall not exceed three hundred sixty (360) days; and
25 c. The respondent to comply with any other reasonable conditions
26 necessary to ensure compliance; or
27 3. The respondent to be released with any reasonable conditions

1 necessary to ensure the safety of self, family, or others and avoid
2 readmittance into a hospital setting; or

3 ~~(c)(10)~~ ~~[If the court finds]~~There is no probable cause to believe the respondent:

4 1. Should be involuntarily hospitalized; or

5 2. Is an individual with a mental illness:

6 a. Who presents a danger or threat of danger to self, family, or
7 others as a result of the mental illness;

8 b. Who may reasonably benefit from court-ordered outpatient
9 treatment or release with reasonable conditions; and

10 c. For whom court-ordered outpatient treatment or release with
11 reasonable conditions is the least restrictive alternative mode of
12 treatment available;]

13 the proceedings against the respondent shall be dismissed, and the respondent
14 shall be released from any hospitalization~~[holding]~~.

15 (11) If upon completion of the final hearing, the court finds:

16 (a) The respondent should be involuntarily hospitalized;]

17 1. The court shall order the respondent to:

18 a. Be hospitalized in a hospital for a period not to exceed sixty (60)
19 consecutive days from the date of the court order or a period not to
20 exceed three hundred sixty (360) consecutive days from the date of
21 the court order, whatever was the period of time that was requested
22 in the petition; and

23 b. Comply with all conditions of the hospital's discharge plan; and

24 2. a. The county attorney may motion the court at the conclusion of
25 the final hearing to require the hospital to provide a copy of the
26 respondent's discharge plan to the county attorney prior to
27 discharge. If the hospital plans to discharge the respondent prior

1 to the expiration of an order under this subsection, the court
2 shall require the hospital to provide a copy of the respondent's
3 discharge plan no later than seventy-two (72) hours prior to the
4 respondent's discharge.

5 b. If the county attorney believes that the discharge plan does not
6 give the respondent a realistic opportunity to avoid imminent
7 readmittance into an inpatient psychiatric hospital for treatment
8 and is insufficient without court-ordered outpatient treatment or
9 reasonable conditions, the county attorney shall:

10 i. Make an ex parte motion for a discharge review hearing.
11 The court shall conduct the discharge review hearing at the
12 earliest practicable time, consistent with due process and
13 the availability of counsel, and in no event later than seven
14 (7) days after the motion is made, except with the
15 respondent's consent or upon the court's finding that
16 extraordinary circumstances exist and that delay is
17 indispensable to protect the respondent and the community;
18 or

19 ii. If the respondent within the past twelve (12) months has
20 been found incompetent to stand trial in a criminal
21 proceeding and has not been committed under KRS
22 Chapter 202C, the county attorney may make a motion to
23 proceed under Section 16 of this Act.

24 c. Upon completion of the discharge review hearing described in
25 subdivision b.i. of this subparagraph, if the court finds that the
26 respondent is an individual with a mental illness, who presents a
27 danger or threat of danger to self, family, or others as a result of

1 the mental illness, who may reasonably benefit from court-
2 ordered outpatient treatment or release with other reasonable
3 conditions, and for whom court-ordered outpatient treatment or
4 release with other reasonable conditions is the least restrictive
5 alternative mode of treatment available, the court shall order:

6 i. For a respondent who meets the criteria for court-ordered
7 assisted outpatient treatment set forth in Section 20 of this
8 Act, that a treatment plan be developed in accordance with
9 KRS 202A.0817 within forty-eight (48) hours and the
10 respondent to receive court-ordered assisted outpatient
11 treatment under KRS 202A.0811 to 202A.0831;

12 ii. For any other respondent who may benefit from outpatient
13 treatment, a qualified mental health professional to develop
14 a treatment plan within forty-eight (48) hours, the
15 respondent to receive community-based outpatient
16 treatment that shall not exceed three hundred sixty (360)
17 days, and the respondent to comply with any other
18 reasonable conditions necessary to ensure compliance; or

19 iii. The respondent to be released with any reasonable
20 conditions necessary to ensure the safety of self, family, or
21 others and avoid readmittance into a hospital setting;

22 (b) The respondent should not be involuntarily hospitalized but that the
23 respondent is an individual with a mental illness who presents a danger or
24 threat of danger to self, family, or others as a result of the mental illness,
25 who may reasonably benefit from court-ordered outpatient treatment or
26 release with other reasonable conditions, and for whom court-ordered
27 outpatient treatment or release with other reasonable conditions is the least

restrictive alternative mode of treatment available, the court shall order:

1. For a respondent who meets the criteria for court-ordered assisted outpatient treatment set forth in Section 20 of this Act:

a. That a treatment plan be developed in accordance with KRS 202A.0817 within forty-eight (48) hours; and

b. The respondent to receive court-ordered assisted outpatient treatment under KRS 202A.0811 to 202A.0831;

2. For any other respondent who may benefit from outpatient treatment:

a. A qualified mental health professional to develop a treatment plan within forty-eight (48) hours;

b. The respondent to receive community-based outpatient treatment that shall not exceed three hundred sixty (360) days; and

c. The respondent to comply with any other reasonable conditions necessary to ensure compliance; or

3. The respondent to be released with any reasonable conditions necessary to ensure the safety of self, family, or others and avoid readmittance into a hospital setting; or

(c) The respondent:

1. Should not be involuntary hospitalized; or

2. Is not an individual with a mental illness:

a. Who presents a danger or threat of danger to self, family, or others as a result of the mental illness;

b. Who may reasonably benefit from court-ordered outpatient treatment or release with reasonable conditions; and

c. For whom court-ordered outpatient treatment or release with reasonable conditions is the least restrictive mode of treatment available;

1 the proceedings against the respondent shall be dismissed, and the
2 respondent shall be released from any hospitalization.

3 (12) (a) Failure to comply with an order for outpatient treatment or release with
4 conditions under subsection (10) or (11) of this section shall not be grounds
5 to find the respondent in contempt of court but shall be prima facie evidence
6 that:

7 1. The respondent can benefit from inpatient hospitalization; and

8 2. Inpatient hospitalization is the least restrictive mode of treatment.

9 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
10 months of an order for outpatient treatment or release with conditions
11 under subsection (10) or (11) of this section, the clerk of the court shall
12 provide the prior order to:

13 1. The court with the petition for involuntary hospitalization; and

14 2. The qualified mental health professional along with the court's order
15 for examination.

16 (c) The qualified mental health professional who conducts the examination as
17 described under paragraph (b)2. of this subsection shall independently
18 evaluate the respondent, considering the respondent's prior order for
19 outpatient treatment or release with conditions, and certify whether he or
20 she meets the criteria for involuntary hospitalization under Section 34 of
21 this Act.

22 (d) A respondent shall not be involuntarily hospitalized based solely on his or
23 her failure to comply with a prior order for outpatient treatment or release
24 with conditions.

25 (13) Any petition under this section shall expire after thirty (30) days if not served
26 upon the respondent.

27 ➔Section 12. KRS 202A.053 is amended to read as follows:

1 (1) (a) Except as provided in paragraph (b) of this subsection, a respondent who has
2 been ordered involuntarily hospitalized following the preliminary hearing
3 shall have venue for all subsequent proceedings, including the final hearing,
4 transferred to the court of the county where the respondent is hospitalized.

5 (b) A court may order venue be transferred back to the county where the
6 respondent resides if the court has ordered the respondent to receive
7 outpatient treatment under Section 11 or 16 of this Act. The receiving
8 county shall then assume venue and responsibility for the respondent's
9 treatment plan and supervision, and shall make orders as the court sees fit.

10 (2) The court of the county where the preliminary hearing was held may, upon its own
11 motion, or shall, upon motion of one (1) of the parties, retain venue over
12 proceedings subsequent to the preliminary hearing.

13 (3) The court of the county where the county attorney has filed a motion for a
14 certification review hearing under Section 9, 10, or 13 of this Act shall retain
15 venue over the proceedings.

16 ➔Section 13. KRS 202A.061 is amended to read as follows:

17 (1) Prior to completion of an examination by a qualified mental health professional
18 under this section, the professional shall make a good-faith attempt to contact the
19 petitioner to obtain any additional relevant information necessary to the petition.

20 (2) (a) Prior to the release of a person subject to an examination under this
21 section, the county attorney may make an ex parte motion for a certification
22 review hearing. The court shall review the ex parte motion upon its receipt.

23 (b) If the review indicates that the person presents an imminent threat of
24 danger to self, family, or others, the court shall order the qualified mental
25 health professionals to immediately notify the court if either of the
26 professionals have certified that the person:

27 1. Is an individual with mental illness;

1 2. Presents a danger or threat of danger to self, family, or others as a
2 result of the mental illness; and

3 3. Does not meet all the criteria for involuntary hospitalization under
4 Section 34 of this Act.

5 (c) The court shall review the certification under paragraph (b) of this
6 subsection and consider if the person has been the subject of proceedings
7 under this chapter, prior to the current proceeding, and is exhibiting an
8 escalation of dangerous behavior. After review, the court may:

9 1. Order the person to be hospitalized in a place designated by the
10 cabinet until the certification review hearing is held within forty-eight
11 (48) hours, excluding weekends and holidays;

12 2. Release the person, subject to reasonable conditions, and set the
13 certification review hearing to be held within forty-eight (48) hours; or

14 3. Deny the county attorney's motion for a certification review and
15 authorize the release of the person.

16 (d) 1. At the certification review hearing, the person shall be appointed
17 counsel who may present evidence and cross examine witnesses on the
18 person's behalf.

19 2. The qualified mental health professional that performed the
20 examination shall testify about the certification under paragraph (b)
21 of this subsection. This testimony may be given remotely.

22 3. Notwithstanding subparagraph 2. of this paragraph, the court may
23 accept the certification under paragraph (b) of this subsection in lieu
24 of testimony upon agreement of all parties and stipulation of the
25 person that he or she meets the criteria for involuntary hospitalization
26 under Section 34 of this Act, provided that outpatient treatment is
27 determined to be the least restrictive mode of treatment.

1 (e) Upon conclusion of the certification review hearing, if the court finds by
2 clear and convincing evidence that the person is an individual with a mental
3 illness, who presents a danger or threat of danger to self, family, or others
4 as a result of the mental illness, who may reasonably benefit from court-
5 ordered outpatient treatment or release with reasonable conditions, and for
6 whom court-ordered outpatient treatment or release with reasonable
7 conditions is the least restrictive alternative mode of treatment available, the
8 court may order the person to:

- 9 1. Receive community-based outpatient treatment that shall not exceed
10 three hundred sixty (360) days and comply with any other reasonable
11 conditions necessary to ensure compliance; or
12 2. Be released with any reasonable conditions necessary to ensure the
13 safety of self, family, or others.

14 (3) (a) Failure to comply with an order for community-based outpatient treatment
15 or release with conditions under subsection (2)(e) of this section shall not be
16 grounds to find the person in contempt of court but shall be prima facie
17 evidence that:

- 18 1. The person can benefit from inpatient hospitalization; and
19 2. Inpatient hospitalization is the least restrictive mode of treatment.

20 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
21 months of an order for community-based outpatient treatment or release
22 with conditions under subsection (2)(e) of this section, the clerk of the court
23 shall provide the prior order to:

- 24 1. The court with the petition for involuntary hospitalization; and
25 2. The qualified mental health professional along with the court's order
26 for examination.

27 (c) The qualified mental health professional who conducts the examination as

1 described under paragraph (b)2. of this subsection shall independently
2 evaluate the person, considering the person's prior order for community-
3 based outpatient treatment or release with conditions, and certify whether
4 he or she meets the criteria for involuntary hospitalization under Section 34
5 of this Act.

6 (d) A person shall not be involuntarily hospitalized based solely on his or her
7 failure to comply with a prior order for community-based outpatient
8 treatment or release with conditions.

9 (4) (a) In any proceeding for involuntary hospitalization under the applicable
10 provisions of this chapter, if the criteria for involuntary hospitalization are not
11 certified by at least two (2) examining qualified mental health professionals,
12 the court shall, without taking any further action, terminate the proceedings
13 and order the release of the person.

14 (b) The qualified mental health professionals shall certify to the court within
15 twenty-four (24) hours, ~~{(excluding weekends and holidays.)}~~ of the
16 examination, their findings and opinions as to whether the person shall be
17 involuntarily hospitalized.

18 ➔SECTION 14. A NEW SECTION OF KRS CHAPTER 202A IS CREATED
19 TO READ AS FOLLOWS:

20 (1) If the court orders community-based outpatient mental health treatment under
21 this chapter, the court shall:

22 (a) Appoint an outpatient provider agency recognized by the cabinet which
23 shall assemble a multidisciplinary team; and

24 (b) Report every order for community-based outpatient treatment issued under
25 this section to the Department for Behavioral Health, Developmental and
26 Intellectual Disabilities.

27 (2) (a) The multidisciplinary team shall:

1 1. Regularly monitor the person's adherence to the conditions of the
2 order and regularly report this information to the court, the county
3 attorney, respondent's counsel, and any other party the court deems
4 necessary; and

5 2. Consist of three (3) mental health professionals, including any of the
6 following:

7 a. The respondent's doctor;

8 b. A nurse practitioner;

9 c. A prescriber;

10 d. A therapist;

11 e. A case manager;

12 f. A peer support specialist; or

13 g. Any other person deemed qualified by the court.

14 (b) Any responsible adult may report nonadherence to the conditions of the
15 order to the court, the county attorney, respondent's counsel, and any other
16 party the court deems necessary.

17 (3) Reports may be provided in written format, in person, or via electronic means, at
18 the court's discretion.

19 (4) The cabinet shall promulgate administrative regulations in accordance with KRS
20 Chapter 13A necessary to implement this section.

21 ➔SECTION 15. A NEW SECTION OF KRS CHAPTER 202A IS CREATED
22 TO READ AS FOLLOWS:

23 (1) A person's substantial failure to comply with a court order for community-based
24 outpatient treatment may constitute presumptive grounds for the court or an
25 authorized staff physician to order a seventy-two (72) hour emergency admission
26 to allow for reexamination of the person to determine whether he or she meets
27 the criteria for involuntary hospitalization under Section 34 of this Act.

1 (2) (a) Failure to comply with an order for community-based outpatient treatment
2 shall not be grounds to find the person in contempt of court but shall be
3 prima facie evidence that:

4 1. The person can benefit from inpatient hospitalization; and

5 2. Inpatient hospitalization is the least restrictive mode of treatment.

6 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
7 months of an order for community-based outpatient treatment, the clerk of
8 the court shall provide the prior order to:

9 1. The court with the petition for involuntary hospitalization; and

10 2. The qualified mental health professional along with the court's order
11 for examination.

12 (c) The qualified mental health professional who conducts the examination as
13 described under paragraph (b)2. of this subsection shall independently
14 evaluate the person, considering the person's prior order for community-
15 based outpatient treatment, and certify whether he or she meets the criteria
16 for involuntary hospitalization under Section 34 of this Act.

17 (d) A person shall not be involuntarily hospitalized based solely on his or her
18 failure to comply with a prior order for community-based outpatient
19 treatment.

20 (3) (a) Any person admitted to a hospital under subsection (1) of this section or
21 transferred to a hospital while ordered hospitalized under subsection (1) of
22 this section shall be transported from the person's home county by the
23 sheriff of that county or other peace officer as ordered by the court.

24 (b) The sheriff or other peace officer may, upon agreement of a person
25 authorized by the peace officer, authorize the cabinet, a private agency on
26 contract with the cabinet, or an ambulance service designated by the cabinet
27 to transport the person to the hospital.

1 (c) The transportation costs of the sheriff, other peace officer, ambulance
2 service, or other private agency on contract with the cabinet shall be paid by
3 the cabinet in accordance with an administrative regulation promulgated by
4 the cabinet in accordance with KRS Chapter 13A.

5 (4) (a) Any person released from the hospital under subsection (1) of this section
6 shall be transported to the person's county of discharge by a sheriff or other
7 peace officer, by an ambulance service designated by the cabinet, or by
8 other appropriate means of transportation which is consistent with the
9 treatment plan of that person.

10 (b) The transportation cost of transporting the patient to the patient's county of
11 discharge when performed by a peace officer, ambulance service, or other
12 private agency on contract with the cabinet shall be paid by the cabinet in
13 accordance with an administrative regulation promulgated by the cabinet in
14 accordance with KRS Chapter 13A.

15 ➔SECTION 16. A NEW SECTION OF KRS CHAPTER 202A IS CREATED
16 TO READ AS FOLLOWS:

17 (1) (a) Upon motion of the county attorney under subsection (11)(a)2.b.ii. of
18 Section 11 of this Act, the hospital shall provide a copy of the discharge
19 plan as soon as practicable to the court and the respondent's counsel of
20 record. The court, upon motion of the county attorney or the court's own
21 motion, shall conduct a review hearing:

22 1. To determine if the discharge plan gives the respondent a realistic
23 opportunity to avoid imminent readmittance into an inpatient
24 psychiatric hospital for treatment; and

25 2. At the earliest practicable time, consistent with due process and the
26 availability of counsel, and in no event later than seven (7) days after
27 the motion is made, except with the respondent's consent or upon the

1 court's finding that extraordinary circumstances exist and that delay is
2 indispensable to protect the respondent and the community.

3 **(b) This hearing shall only be conducted for a respondent who within the past**
4 twelve (12) months has been found incompetent to stand trial in a criminal
5 proceeding and has not been committed under KRS Chapter 202C.

6 **(2) The court shall verify that discharge planning procedures were completed to give**
7 the respondent a realistic opportunity to avoid recurrence of substantial symptom
8 burden that would necessitate psychiatric hospitalization. Those procedures shall
9 include but not be limited to any of the following:

10 **(a) Documenting the housing status of the respondent or that housing services**
11 were offered and the respondent declined;

12 **(b) Scheduling an outpatient treatment appointment for no later than seven (7)**
13 days after discharge;

14 **(c) Providing or prescribing a thirty (30) day supply of medication;**

15 **(d) Documenting a transportation plan that may include securing a bus pass,**
16 taxi voucher, or an acknowledgment that the respondent will walk or the
17 respondent's family will provide transportation;

18 **(e) Developing and documenting a crisis plan with contact information for**
19 services that are available twenty-four (24) hours a day;

20 **(f) Developing a transfer of care plan or attempting to develop a transfer of**
21 care plan with the respondent if the respondent has met with an outpatient
22 provider or an appointment with the outpatient provider has been
23 scheduled; or

24 **(g) Contacting the respondent's family or other support systems, if the**
25 respondent consents.

26 **(3) For respondents with decision-making capacity who refuse housing or other**
27 services under subsection (2) of this section, the discharge plan shall document:

- 1 (a) That housing and other services were offered and explained;
2 (b) The respondent's reasons for refusal;
3 (c) That the respondent understands the potential consequences of his or her
4 choices;
5 (d) That risk reduction strategies were offered, including access to crisis
6 contacts, mobile crisis services, drop-in services, and other safety-oriented
7 resources; and
8 (e) That there is a plan for periodic outreach to offer services.
9 (4) If the court finds by clear and convincing evidence that the discharge plan does
10 not give the respondent a realistic opportunity to avoid imminent readmittance
11 into an inpatient psychiatric hospital for treatment, then the respondent shall not
12 be discharged and the court shall order the hospital to submit within seventy-two
13 (72) hours a revised discharge plan that gives the respondent a realistic
14 opportunity to avoid imminent readmittance into an inpatient psychiatric hospital
15 for treatment.
16 (5) (a) If a respondent is not discharged under subsection (4) of this section, a
17 review hearing shall be conducted by the court within seven (7) days of the
18 hospital's submission of a revised discharge plan.
19 (b) A review hearing may be conducted in an informal manner, consistent with
20 orderly procedures, and in a physical setting not likely to have a harmful
21 effect on the mental or physical health of the respondent. The hearing may
22 be held by the court in chambers, remotely from a hospital, or in another
23 suitable place. The respondent shall be present in person or remotely for all
24 review hearings, unless presence is waived by the respondent through
25 counsel.
26 (c) 1. The Commonwealth shall present evidence regarding whether:
27 a. The respondent continues to meet the criteria for involuntary

- 1 commitment under Section 34 of this Act; and
- 2 b. The discharge plan gives the respondent a realistic opportunity
3 to avoid imminent readmittance into an inpatient psychiatric
4 hospital for treatment.
- 5 2. The respondent shall be represented by counsel and be afforded an
6 opportunity to present evidence and to cross-examine any witnesses.
- 7 (d) The Kentucky Rules of Evidence shall apply and proceedings shall be heard
8 by a judge without a jury.
- 9 (e) The respondent's right to this hearing shall not be waived.
- 10 (f) 1. At the conclusion of a review hearing, the court shall make written
11 findings of fact concerning whether:
- 12 a. The criteria for involuntary commitment under Section 34 of this
13 Act continue to be satisfied; and
- 14 b. The discharge plan gives the respondent a realistic opportunity
15 to avoid imminent readmittance into an inpatient psychiatric
16 hospital for treatment.
- 17 2. If the court finds, by clear and convincing evidence, that the criteria
18 continue to be satisfied and that the discharge plan does not give the
19 respondent a realistic opportunity to avoid imminent readmittance into
20 an inpatient psychiatric hospital for treatment, the court shall enter an
21 order authorizing the continued care and treatment of the respondent
22 until the expiration of the order under subsection (11)(a) of Section 11
23 of this Act.
- 24 3. If the court does not make the finding required in subparagraph 2. of
25 this paragraph, the court shall discharge the respondent to comply
26 with the discharge plan unless the court finds that the respondent is
27 an individual with a mental illness, who presents a danger or threat of

1 danger to self, family, or others as a result of the mental illness, who
2 may reasonably benefit from court-ordered outpatient treatment or
3 release with other reasonable conditions, and for whom court-ordered
4 outpatient treatment or release with other reasonable conditions is the
5 least restrictive alternative mode of treatment available, in which case
6 the court shall discharge the respondent and may order:

7 a. For a respondent who meets the criteria for court-ordered
8 assisted outpatient treatment set forth in Section 20 of this Act:

9 i. That at a treatment plan be developed in accordance with
10 Section 20 of this Act within forty-eight (48) hours; and

11 ii. The respondent to receive court-ordered assisted outpatient
12 treatment under KRS 202A.0811 to 202A.0831;

13 b. For any other respondent who may benefit from outpatient
14 treatment:

15 i. A qualified mental health professional to develop a
16 treatment plan within forty-eight (48) hours;

17 ii. The respondent to receive community-based outpatient
18 treatment that shall not exceed three hundred sixty (360)
19 days; and

20 iii. The respondent to comply with any other reasonable
21 condition necessary to ensure compliance; or

22 c. The respondent to be released with any reasonable conditions
23 necessary to ensure the safety of self, family, or others and avoid
24 readmittance into a hospital setting.

25 (6) The Commonwealth, respondent, or hospital where the respondent is being
26 hospitalized may make a motion for an additional review hearing if a material
27 change in circumstances has occurred and the respondent no longer meets the

criteria for involuntary hospitalization under Section 34 of this Act. If the court has probable cause to believe that a material change in circumstances has occurred, the court shall:

(a) Conduct a review hearing within fourteen (14) days of the filing of the motion for an additional review hearing; and

(b) Order the respondent to be evaluated as described under subsection (5)(b) of this section.

(7) (a) Any respondent being hospitalized under this section shall be transferred to an inpatient psychiatric hospital owned by the Commonwealth upon request of the hospital or psychiatric facility where the respondent is being held.

(b) An inpatient psychiatric hospital contracted with the cabinet shall not request transfer under paragraph (a) of this subsection.

➔SECTION 17. A NEW SECTION OF KRS CHAPTER 202A IS CREATED TO READ AS FOLLOWS:

(1) By October 1 of each year beginning in 2027, the cabinet in coordination with the Administrative Office of the Courts shall submit to the Legislative Research Commission for referral to the Interim Joint Committee on Health Services and the Interim Joint Committee on Judiciary a report that includes:

(a) The number of:

1. Petitions filed under Section 11 of this Act;

2. Petitions filed under Section 19 of this Act;

3. Court orders for seventy-two (72) hour hospital admission;

4. Petitions dismissed prior to a final order under Section 11 of this Act or KRS 202A.0819;

5. Petitions that result in an order for involuntary hospitalization under Section 11 of this Act and if the orders were for sixty (60) days or three hundred sixty (360) days of involuntary hospitalization;

- 1 6. Petitions that result in an order for outpatient treatment or any other
2 reasonable conditions;
- 3 7. Petitions filed following a respondent's violation of court-ordered
4 outpatient treatment or other reasonable conditions;
- 5 8. Precertification review hearings conducted under this chapter;
- 6 9. Discharge hearings conducted under Section 11 of this Act; and
- 7 10. Review hearings conducted under Section 16 of this Act;
- 8 (b) The stage of a proceeding under this chapter where a:
- 9 1. Petition is dismissed; and
- 10 2. Court has ordered a respondent to court-ordered outpatient or any
11 other reasonable conditions;
- 12 (c) The length of time a respondent receives treatment under this chapter prior
13 to discharge by a hospital or termination of an order to receive outpatient
14 treatment; and
- 15 (d) A summary of the services provided to a respondent who is ordered to
16 involuntary hospitalization or to receive outpatient treatment.
- 17 (2) Any hospital, qualified mental health professional, and any other mental health
18 agency who evaluates or treats a respondent under this chapter shall be required
19 to report data to the cabinet as required under this section. The cabinet shall
20 promulgate administrative regulations in accordance with KRS Chapter 13A to
21 implement this section.
- 22 (3) The cabinet and the Administrative Office of the Courts shall enter into an
23 agreement to share data necessary to prepare the report required under this
24 section.
- 25 (4) The report prepared under this section shall not identify specific individuals.

26 ➔SECTION 18. A NEW SECTION OF KRS CHAPTER 202A IS CREATED
27 TO READ AS FOLLOWS:

1 (1) In any proceeding under this chapter, if the respondent agrees:

2 (a) To comply with a treatment plan; and

3 (b) That he or she meets the criteria for involuntary hospitalization under
4 Section 34 of this Act, provided that outpatient treatment is determined to be
5 the least restrictive mode of treatment; and

6 upon agreement of the court, county attorney, respondent, respondent's counsel,
7 and qualified mental health professional, the court may enter a consent order for
8 the respondent to receive court-ordered assisted outpatient treatment or
9 community-based outpatient treatment and order other reasonable conditions.

10 (2) An order entered under subsection (1) of this section shall:

11 (a) Be filed with the court and made a part of the record in the proceeding;

12 (b) Specify the terms and conditions of treatment to which the respondent has
13 agreed to;

14 (c) Specify that the respondent is subject to same monitoring, reporting, and
15 review requirements as any contested order to receive court-ordered assisted
16 outpatient treatment or community-based outpatient treatment; and

17 (d) Remain in effect for a period not to exceed three hundred sixty (360) days,
18 subject to early termination or modification by the court.

19 (3) The failure of a respondent to comply with a consent order entered under this
20 section shall be treated in the same manner as noncompliance with a contested
21 order under this chapter.

22 ➔Section 19. KRS 202A.0811 is amended to read as follows:

23 (1) Proceedings for court-ordered assisted outpatient treatment of a person shall be
24 initiated by the filing of a verified petition for that purpose in District Court.

25 (2) The petition and all subsequent court documents shall be entitled: "In the interest of
26 (name of respondent)."

27 (3) The petition shall be filed by a qualified mental health professional; peace officer;

1 county attorney; Commonwealth's attorney; spouse, relative, friend, or guardian of
2 the person concerning whom the petition is filed; or any responsible adult~~other~~
3 ~~interested person~~].

- 4 (4) The petition shall set forth:
- 5 (a) Petitioner's relationship to the respondent;
 - 6 (b) Respondent's name, residence, and current location, if known;
 - 7 (c) Petitioner's belief, including the factual basis therefor, that the respondent
8 meets the criteria for court-ordered assisted outpatient treatment as set forth in
9 KRS 202A.0817; and
 - 10 (d) Whether, within five (5) days prior to the filing of the petition, the respondent
11 has been evaluated by a qualified mental health professional to determine
12 whether the respondent meets the criteria for court-ordered assisted outpatient
13 treatment pursuant to KRS 202A.0815.
- 14 (5) Upon receipt of the petition, the court shall examine the petitioner under oath as to
15 the contents of the petition. If the petitioner is a qualified mental health
16 professional, the court may dispense with the examination.
- 17 (6) If, after reviewing the allegations contained in the petition and examining the
18 petitioner under oath, it appears to the court that there is probable cause to believe
19 the respondent should be court-ordered to assisted outpatient treatment, the court
20 shall:
- 21 (a) Order the respondent to be evaluated without unnecessary delay by a qualified
22 mental health professional to determine whether the respondent meets the
23 criteria for court-ordered assisted outpatient treatment set forth in KRS
24 202A.0815, unless the court has already received the certified findings of such
25 an evaluation conducted no earlier than five (5) days prior to the filing of the
26 petition. The qualified mental health professional shall certify his or her
27 findings to the court within seventy-two (72) hours from receipt of the order,

1 excluding weekends and holidays; and

2 (b) Set a date for a hearing within six (6) days from the date of the filing of the
3 petition under the provisions of this section, excluding weekends and
4 holidays, to determine if the respondent should be court-ordered to assisted
5 outpatient treatment.

6 (7) If the court finds there is no probable cause to believe the respondent should be
7 court-ordered to assisted outpatient treatment, the proceedings against the
8 respondent shall be dismissed.

9 ➔Section 20. KRS 202A.0815 is amended to read as follows:

10 ~~A[No]~~ person shall **not** be court-ordered to assisted outpatient mental health treatment
11 unless the person:

12 (1) Is diagnosed with a serious mental illness;

13 (2) Has a history of repeated nonadherence with mental health treatment, which has:

14 (a) At least twice within the last forty-eight (48) months, been a significant factor
15 in necessitating hospitalization or arrest of the person; or

16 (b) Within the last twenty-four (24) months, resulted in an act, threat, or attempt
17 at serious physical injury to self or others;

18 (3) Is unlikely to adequately adhere to outpatient treatment on a voluntary basis
19 based on a qualified mental health professional's ~~[-~~

20 ~~(a)]clinical observation[-; and~~

21 ~~(b) Identification of specific characteristics of the person's clinical condition that~~
22 ~~significantly impair the person's ability to make and maintain a rational and~~
23 ~~informed decision as to whether to engage in outpatient treatment~~
24 ~~voluntarily]; and~~

25 (4) Is in need of court-ordered assisted outpatient treatment as the least restrictive
26 alternative mode of treatment presently available and appropriate.

27 ➔Section 21. KRS 202A.0819 is amended to read as follows:

- 1 (1) At a hearing and at all stages of a proceeding for court-ordered assisted outpatient
2 treatment, the respondent shall be:
- 3 (a) Represented by counsel;
- 4 (b) Accompanied by a peer support specialist or other person in a support
5 relationship, if requested by the respondent; and
- 6 (c) Afforded an opportunity to present evidence, call witnesses on his or her
7 behalf, and cross-examine adverse witnesses.
- 8 (2) If a respondent does not appear at the hearing, and appropriate attempts to elicit the
9 respondent's appearance have failed, the court may conduct the hearing in the
10 respondent's absence.
- 11 (3) A qualified mental health professional who recommends court-ordered assisted
12 outpatient treatment for the respondent shall:
- 13 (a) Testify at the hearing, in person or via electronic means;
- 14 (b) State the facts and clinical determinations which support the allegation that
15 the respondent meets the criteria stated in KRS 202A.0815; and
- 16 (c) Testify in support of the treatment plan provided pursuant to KRS 202A.0817,
17 and for each category of proposed evidence-based treatment, he or she shall
18 state the specific recommendation and the clinical basis for his or her belief
19 that such treatment is essential to the maintenance of the respondent's health
20 or safety.
- 21 (4) If after hearing all relevant evidence, the court does not find by clear and
22 convincing evidence that the respondent meets the criteria stated in KRS
23 202A.0815, the court shall deny the petition and the proceedings against the
24 respondent shall be dismissed.
- 25 (5) If after hearing all relevant evidence, the court finds by clear and convincing
26 evidence that the respondent meets the criteria stated in KRS 202A.0815, the court
27 may order the respondent to receive assisted outpatient treatment for a period of

1 time not to exceed three hundred sixty (360) days and to comply with any other
2 reasonable conditions necessary to ensure compliance. The court's order shall
3 incorporate a treatment plan, which shall be limited in scope to the
4 recommendations included in the treatment plan provided by the qualified mental
5 health professional pursuant to KRS 202A.0817.

6 (6) The court shall report every order for assisted outpatient treatment issued under this
7 section to the Department for Behavioral Health, Developmental and Intellectual
8 Disabilities.

9 ➔Section 22. KRS 202A.0823 is amended to read as follows:

10 (1) A person's substantial failure to comply with a court order for assisted outpatient
11 treatment may constitute presumptive grounds for the court or an authorized staff
12 physician to order a seventy-two (72) hour emergency admission to allow for
13 reexamination of the person to determine whether he or she meets the criteria for
14 involuntary hospitalization under Section 34 of this Act~~[pursuant to KRS~~
15 ~~202A.031]~~.

16 (2) (a) Upon the refusal of a person subject to a court order for assisted outpatient
17 treatment to participate in any or all aspects of his or her treatment plan,
18 the person's outpatient provider may establish a review committee that is
19 made up of three (3) qualified mental health professionals to examine the
20 appropriateness of the person's treatment plan. The review committee shall
21 include at least one (1) qualified mental health professional who is directly
22 involved with the person's treatment, if available. Within three (3) days of
23 the refusal, the review committee shall meet the person and his or her
24 counsel or other representative to discuss its recommendations.

25 (b) If the person still refuses to participate in any or all aspects of his or her
26 treatment plan, the person's outpatient provider may petition the District
27 Court for a de novo determination of the appropriateness of the proposed

1 treatment.

2 (c) Within seven (7) days, the court shall conduct a hearing, consistent with the
3 person's rights to due process of law, and shall utilize the following factors
4 in reaching its determination:

5 1. Whether the treatment is necessary to protect the person or others
6 from harm;

7 2. Whether the person is incapable of giving informed consent to the
8 proposed treatment;

9 3. Whether any less restrictive alternative treatment exists; and

10 4. Whether the proposed treatment carries any risk of permanent side
11 effects.

12 (d) 1. Upon completion of the hearing, the court shall enter an appropriate
13 judgment.

14 2. An order entered under this paragraph shall not allow for a person to
15 be physically restrained in order to administer any medication in any
16 outpatient treatment setting.

17 (e) A judgment entered under paragraph (d) of this subsection may extend to
18 treatment provided during a seventy-two (72) hour emergency admission.

19 (3) (a) Failure to comply with an order for assisted outpatient treatment shall not be
20 grounds to find the person in contempt of court but shall be prima facie
21 evidence that:

22 1. The person can benefit from inpatient hospitalization; and

23 2. Inpatient hospitalization is the least restrictive mode of treatment.

24 (b) If a new petition for involuntary hospitalization is filed within twelve (12)
25 months of an order for assisted outpatient treatment, the clerk of the court
26 shall provide the prior order to:

27 1. The court with the petition for involuntary hospitalization; and

1 2. The qualified mental health professional along with the court's order
2 for examination.

3 (c) The qualified mental health professional who conducts the examination as
4 described under paragraph (b)2. of this subsection shall independently
5 evaluate the person, considering the person's prior order for assisted
6 outpatient treatment, and certify whether he or she meets the criteria for
7 involuntary hospitalization under Section 34 of this Act.

8 (d) A person shall not be involuntarily hospitalized based solely on his or her
9 failure to comply with a prior order for assisted outpatient treatment.

10 (4) (a) Any person admitted to a hospital under subsection (1) of this section or
11 transferred to a hospital while ordered hospitalized under subsection (1) of
12 this section shall be transported from the person's home county by the
13 sheriff of that county or other peace officer as ordered by the court.

14 (b) The sheriff or other peace officer may, upon agreement of a person
15 authorized by the peace officer, authorize the cabinet, a private agency on
16 contract with the cabinet, or an ambulance service designated by the cabinet
17 to transport the person to the hospital.

18 (c) The transportation costs of the sheriff, other peace officer, ambulance
19 service, or other private agency on contract with the cabinet shall be paid by
20 the cabinet in accordance with an administrative regulation promulgated by
21 the cabinet in accordance with KRS Chapter 13A.

22 (5) (a) Any person released from the hospital under subsection (1) of this section
23 shall be transported to the person's county of discharge by a sheriff or other
24 peace officer, by an ambulance service designated by the cabinet, or by
25 other appropriate means of transportation which is consistent with the
26 treatment plan of that person.

27 (b) The transportation cost of transporting the patient to the patient's county of

1 discharge when performed by a peace officer, ambulance service, or other
2 private agency on contract with the cabinet shall be paid by the cabinet in
3 accordance with an administrative regulation promulgated by the cabinet in
4 accordance with KRS Chapter 13A.

5 ➔Section 23. KRS 202A.091 is amended to read as follows:

6 (1) The court records of a respondent made in all proceedings pursuant to this[KRS]
7 chapter [202A]are hereby declared to be confidential and shall not be open to the
8 general public for inspection except when such disclosure is provided in KRS
9 202A.016.

10 (2) Twelve (12) months after[following] the discharge of a respondent from a
11 treatment facility or the issuance of a court order denying a petition for a
12 commitment, a respondent may [at any time]move to have all court records
13 pertaining to the proceedings expunged from the files of the court. The county
14 attorney shall be given notice of the[any such] motion and shall have five (5) days
15 in which to respond to the motion[same] or request a hearing on the
16 motion[thereon].

17 (3) (a) Any petitioner under Section 11 of this Act who qualifies as a responsible
18 party under KRS 311.631 may motion the court to participate in court
19 proceedings and be informed by a hospital of the discharge plan prior to a
20 respondent's release under this chapter, except for any confidential
21 therapeutic communication or any other medical records.

22 (b) The court may enter a written order allowing the petitioner to participate as
23 described in paragraph (a) of this subsection, unless the court finds the
24 petitioner's participation is not in the best interests of the respondent.

25 (4) Any person seeking information contained in the court files or the court records of
26 proceedings involving persons under this chapter may file a written motion in the
27 cause setting out why the information is needed. A District Judge may issue an

1 order to disclose the information sought if he or she finds ~~the~~^{such} order is
2 appropriate under the circumstances and if he or she finds it is in the best interest of
3 the person or of the public to have ~~the~~^{such} information disclosed.

4 ➔Section 24. KRS 202A.101 is amended to read as follows:

5 (1) The court which orders any person to the receiving hospital or psychiatric facility,
6 under the provisions of this chapter~~,~~ shall immediately~~at once~~ notify the
7 receiving hospital or psychiatric facility that ~~the~~^{such} order has been made,
8 advising of the sex and condition of the person.

9 (2) After the facility has been ~~so~~ notified, the court shall order the sheriff of the
10 county or other peace officer to transport the patient within forty-eight (48) hours,
11 ~~{excluding weekends and holidays.}~~ from the county in which the person is
12 located to the hospital or psychiatric facility designated by the cabinet. The sheriff
13 or other peace officer may, upon agreement of a person authorized by the peace
14 officer, authorize the cabinet, a private agency on contract with the cabinet, or an
15 ambulance service designated by the cabinet to transport the person to the hospital.

16 (3) The transportation costs of transporting a person to a hospital or psychiatric facility,
17 when performed by a peace officer, an ambulance service, or other private agency
18 on contract with the cabinet shall be paid by the cabinet in accordance with
19 administrative regulation promulgated by the cabinet under the provisions of KRS
20 Chapter 13A.

21 (4) In returning any patient to the county from which the patient is sent, the
22 transportation cost of the sheriff or other peace officer, the ambulance service, or
23 the other agency on contract with the cabinet transporting the patient shall be paid
24 as provided in KRS 202A.028~~(4)~~⁽⁵⁾, when necessary.

25 (5) Whenever an individual is involuntarily hospitalized by a court order the patient
26 shall be transported to the hospital designated by the cabinet and accompanied by
27 the following documents:

1 (a) A copy of the petition for involuntary hospitalization, unless hospitalization
2 takes place pursuant to KRS 202A.041 **or Section 15 or 22 of this Act;**

3 (b) The certificate of qualified mental health professionals, **if any;** and

4 (c) The order of involuntary hospitalization.

5 (6) The hospital may refuse to receive any person who has been ordered to be
6 involuntarily hospitalized by a court order if the papers presented with ~~the~~^[such]
7 person at the hospital do not comply with the provisions of this chapter or if it does
8 not receive notification of the order of involuntary hospitalization as required by
9 this chapter.

10 ➔Section 25. KRS 202A.171 is amended to read as follows:

11 **Except as provided otherwise in this chapter,** an authorized staff physician of a hospital
12 shall discharge an involuntary patient when he no longer meets the criteria for
13 involuntary hospitalization.

14 ➔Section 26. KRS 202C.010 is amended to read as follows:

15 As used in this chapter, unless the context otherwise requires:

16 (1) "Cabinet" means the ~~[Kentucky]~~Cabinet for Health and Family Services;

17 (2) "Commitment hearing" means the hearing under KRS 202C.040 to determine if a
18 respondent meets the criteria for involuntary commitment under this chapter;

19 (3) "Danger" means substantial physical harm or threat of substantial physical harm
20 upon self or others;

21 (4) "Evidentiary hearing" means the hearing under KRS 202C.030 to determine if the
22 **respondent is responsible for**~~[defendant committed]~~ the qualifying offense for
23 which he or she was charged by a preponderance of the evidence;

24 (5) "Forensic psychiatric facility" means a mental institution or facility, or part thereof,
25 designated by the secretary for the purpose and function of providing inpatient
26 evaluation, care, and treatment for ~~[mentally ill persons or]~~ individuals with an
27 intellectual disability **or mental illness** who have been charged with or convicted of

1 a felony;

2 (6) "Hospital" means:

3 (a) A state mental hospital or institution or other licensed public or private
4 hospital, institution, health-care facility, or part thereof, approved by the
5 cabinet~~[Kentucky Cabinet for Health and Family Services]~~ as equipped to
6 provide full-time residential care and treatment for ~~[mentally ill persons or~~
7 ~~]individuals with an intellectual disability~~ **or mental illness**; or

8 (b) A hospital, institution, or health-care facility of the government of the United
9 States equipped to provide residential care and treatment for ~~[mentally ill~~
10 ~~persons or]individuals with an intellectual disability~~ **or mental illness**;

11 (7) **"Individual with a mental illness" means a person with substantially impaired**
12 **capacity to use self-control, judgment, or discretion in the conduct of the person's**
13 **affairs and social relations, associated with maladaptive behavior or recognized**
14 **emotional symptoms where impaired capacity, maladaptive behavior, or**
15 **emotional symptoms can be related to physiological, psychological, or social**
16 **factors**;

17 **(8)** "Individual with an intellectual disability" means a person with significantly
18 subaverage general intellectual functioning existing concurrently with deficits in
19 adaptive behavior and manifested during the developmental period;

20 **(9)**~~(8)~~ "Judge" means the judge who found the respondent incompetent to stand trial
21 in the criminal proceeding from which the petition for involuntary commitment
22 arose;

23 **(10)**~~(9)~~ "Less restrictive alternative mode of treatment" means a treatment given
24 outside of a forensic psychiatric facility which would provide a respondent with
25 appropriate treatment or care consistent with accepted professional practice
26 standards and protect the respondent's safety and the safety of others;{

27 ~~(10) "Mentally ill person" means a person with substantially impaired capacity to use~~

1 self-control, judgment, or discretion in the conduct of the person's affairs and social
2 relations, associated with maladaptive behavior or recognized emotional symptoms
3 where impaired capacity, maladaptive behavior, or emotional symptoms can be
4 related to physiological, psychological, or social factors;}

5 (11) "Qualified mental health professional" means:

6 (a) A physician licensed under the laws of Kentucky to practice medicine or
7 osteopathy, or a medical officer of the government of the United States while
8 engaged in the performance of official duties;

9 (b) A psychiatrist licensed under the laws of Kentucky to practice medicine or
10 osteopathy, or a medical officer of the government of the United States while
11 engaged in the practice of official duties, who is certified or eligible to apply
12 for certification by the American Board of Psychiatry and Neurology, Inc.;

13 (c) A psychologist with the health service provider designation, a psychological
14 practitioner, a certified psychologist, or a psychological associate, licensed
15 under the provisions of KRS Chapter 319;

16 (d) A licensed registered nurse with a master's degree in psychiatric nursing from
17 an accredited institution and two (2) years of clinical experience with
18 individuals with a mental illness~~[mentally ill persons]~~, or a licensed
19 registered nurse, with a bachelor's degree in nursing from an accredited
20 institution, who is certified as a psychiatric and mental health nurse by the
21 American Nurses Association and who has three (3) years of inpatient or
22 outpatient clinical experience in psychiatric nursing and is currently employed
23 by a hospital or forensic psychiatric facility licensed by the Commonwealth or
24 a psychiatric unit of a general hospital or a private agency or company
25 engaged in the provision of mental health services or a regional community
26 program for mental health and individuals with an intellectual disability;

27 (e) A licensed clinical social worker licensed under the provisions of KRS

1 335.100, or a certified social worker licensed under the provisions of KRS
2 335.080 with three (3) years of inpatient or outpatient clinical experience in
3 psychiatric social work and currently employed by a hospital or forensic
4 psychiatric facility licensed by the Commonwealth or a psychiatric unit of a
5 general hospital or a private agency or company engaged in the provision of
6 mental health services or a regional community program for mental health and
7 individuals with an intellectual disability;

8 (f) A marriage and family therapist licensed under the provisions of KRS
9 335.300 to 335.399 with three (3) years of inpatient or outpatient clinical
10 experience in psychiatric mental health practice and currently employed by a
11 hospital or forensic facility licensed by the Commonwealth, a psychiatric unit
12 of a general hospital, a private agency or company engaged in providing
13 mental health services, or a regional community program for mental health
14 and individuals with an intellectual disability;

15 (g) A professional counselor credentialed under the provisions of KRS Chapter
16 335.500 to 335.599 with three (3) years of inpatient or outpatient clinical
17 experience in psychiatric mental health practice and currently employed by a
18 hospital or forensic facility licensed by the Commonwealth, a psychiatric unit
19 of a general hospital, a private agency or company engaged in providing
20 mental health services, or a regional community program for mental health
21 and individuals with an intellectual disability; or

22 (h) A physician assistant licensed under KRS 311.840 to 311.862, who meets one
23 (1) of the following requirements:

- 24 1. Provides documentation that he or she has completed a psychiatric
25 residency program for physician assistants;
- 26 2. Has completed at least one thousand (1,000) hours of clinical experience
27 under a supervising physician, as defined by KRS 311.840, who is a

1 psychiatrist and is certified or eligible for certification by the American
2 Board of Psychiatry and Neurology, Inc.;

3 3. Holds a master's degree from a physician assistant program accredited
4 by the Accreditation Review Commission on Education for the
5 Physician Assistant or its predecessor or successor agencies, is
6 practicing under a supervising physician as defined by KRS 311.840,
7 and:

8 a. Has two (2) years of clinical experience in the assessment,
9 evaluation, and treatment of mental disorders; or

10 b. Has been employed by a hospital or forensic psychiatric facility
11 licensed by the Commonwealth or a psychiatric unit of a general
12 hospital or a private agency or company engaged in the provision
13 of mental health services or a regional community program for
14 mental health and individuals with an intellectual disability for at
15 least two (2) years; or

16 4. Holds a bachelor's degree, possesses a current physician assistant
17 certificate issued by the board prior to July 15, 2002, is practicing under
18 a supervising physician as defined by KRS 311.840, and:

19 a. Has three (3) years of clinical experience in the assessment,
20 evaluation, and treatment of mental disorders; or

21 b. Has been employed by a hospital or forensic psychiatric facility
22 licensed by the Commonwealth or a psychiatric unit of a general
23 hospital or a private agency or company engaged in the provision
24 of mental health services or a regional community program for
25 mental health and individuals with an intellectual disability for at
26 least three (3) years;

27 (12) "Qualifying offense" means a capital offense, a Class A felony, a Class B felony

1 resulting in death or serious physical injury, or a violation of KRS 510.040 or
2 510.070;

3 (13) "Respondent" means a person who was a criminal defendant found incompetent to
4 stand trial who is or was the subject of a petition for involuntary commitment filed
5 under KRS Chapter 504;

6 (14) "Review hearing" means any hearing conducted to determine if a respondent
7 continues to meet the criteria for involuntary commitment after the initial order for
8 involuntary commitment has been issued under this chapter; and

9 (15) "Secretary" means the secretary of the Cabinet for Health and Family Services.

10 ➔Section 27. KRS 202C.020 is amended to read as follows:

11 (1) When a defendant who is charged with a qualifying offense has been found, after a
12 hearing under KRS Chapter 504, to be incompetent to stand trial with no substantial
13 probability that the defendant will attain competency within three hundred sixty
14 (360) days, the Commonwealth's attorney's office serving the county of criminal
15 prosecution shall immediately petition the Circuit Court that found the defendant
16 incompetent to stand trial or, if the finding was by a District Court, the Circuit
17 Court in the county of the criminal prosecution, for an involuntary commitment
18 proceeding, to include an evidentiary hearing and a commitment hearing, if
19 applicable, under this chapter.

20 (2) (a) Upon the filing of the petition, the court shall assign a guardian ad litem to
21 represent the ~~needs and~~ best interest of the respondent, **independent of the**
22 **respondent's defense attorney.**

23 (b) The guardian ad litem shall:

24 1. Be a full and active participant in all proceedings other than the
25 evidentiary hearing under KRS 202C.030. **At the evidentiary hearing,**
26 **the guardian ad litem's role shall be limited to assisting the respondent**
27 **with decision-making related to the hearing, including but not limited**

1 to whether to waive the hearing, whether to stipulate to the allegations,
2 and whether to testify. The guardian ad litem shall not act as an
3 attorney advocate at the evidentiary hearing;~~and shall~~

4 2. Independently investigate, assess, and advocate for the
5 respondent's~~defendant's~~ best interest;~~[-. The guardian ad litem is]~~

6 3. Not be a replacement for the respondent's defense attorney; and

7 4. Be paid an hourly rate not to exceed the maximum hourly rate
8 provided in the Legal Services Duties and Maximum Rate Schedule
9 promulgated by the Government Contract Review Committee
10 established pursuant to KRS 45A.705, to be paid by the Finance and
11 Administration Cabinet.

12 (c) If the respondent~~defendant~~ has retained or been appointed a defense
13 attorney in the criminal case, that attorney may continue to represent the
14 respondent~~defendant~~ in proceedings under this chapter. If, at any time
15 during the pendency of proceedings under this chapter, the
16 respondent~~defendant~~ is not represented by an attorney, the court shall
17 appoint counsel for the respondent~~defendant~~, without a showing of
18 indigency, to be provided by the Department of Public Advocacy or its
19 designee.

20 (3) The Circuit Court shall have exclusive jurisdiction over all proceedings under this
21 chapter.

22 (4) The Circuit Court in the county of the criminal prosecution shall retain
23 jurisdiction over all proceedings under this chapter until the respondent is
24 discharged.

25 ➔ Section 28. KRS 202C.030 is amended to read as follows:

26 (1) An adversarial evidentiary hearing on the record shall be held within forty-five
27 (45)~~twenty (20)~~ days, excluding weekends and holidays, of the filing of a petition

- 1 pursuant to KRS 202C.020, **unless the court orders a later hearing for good cause**
2 **shown**. Appropriate notice shall be served on all parties. The court shall order the
3 Commonwealth to provide all available discovery to the respondent no later than
4 seven (7) days, excluding weekends and holidays, before the hearing, **unless the**
5 **court orders a later date for good cause shown**. ~~[No]~~ Evidence **that has not been**
6 **disclosed through discovery shall not** ~~[may]~~ be presented at the hearing ~~[that has~~
7 ~~not been disclosed through discovery]~~.
- 8 (2) The respondent may stipulate to potential **responsibility** ~~[guilt]~~ and waive the
9 hearing. A stipulation of potential **responsibility shall not** ~~[guilt cannot]~~ be used
10 against the respondent in any future criminal prosecution or civil litigation.
- 11 (3) **(a)** The purpose of the evidentiary hearing shall be to determine whether
12 sufficient evidence exists to support a finding that the respondent is
13 **responsible for** ~~[guilty of]~~ the charged crime against him or her.
- 14 **(b)** The Commonwealth's attorney's office serving the county of criminal
15 prosecution shall have the burden of proving the sufficiency of the evidence
16 by a preponderance of the evidence.
- 17 (4) The evidentiary hearing shall be held before a judge without a jury. The rules of
18 evidence shall apply. The respondent shall be permitted to present evidence and
19 cross examine witnesses. The respondent may present evidence of affirmative
20 defenses that could be raised at a criminal trial on the charged crime, **except for the**
21 **defense of insanity**. The Commonwealth shall not have the burden of disproving an
22 affirmative defense. The respondent must prove an affirmative defense by a
23 preponderance of the evidence.
- 24 (5) (a) If the court determines that sufficient evidence has been presented to support a
25 finding that the respondent is **responsible for** ~~[guilty of]~~ the charged crime
26 against him or her, the court shall immediately schedule a commitment
27 hearing under this chapter **to be held** within **forty-five (45)** ~~[twenty (20)]~~ days.

1 **unless the court orders a later hearing for good cause shown**, excluding
2 weekends and holidays.

3 (b) **1.** The court shall cause the respondent to be examined without
4 unnecessary delay by two (2) qualified mental health professionals, at
5 least one (1) of whom is a physician. The qualified mental health
6 professionals shall, **no fewer than**~~[within]~~ seven (7) days, excluding
7 weekends and holidays, prior to the hearing, certify to the court their
8 findings as to whether the respondent meets the criteria for involuntarily
9 commitment under KRS 202C.050.

10 **2. A copy of the findings submitted under subparagraph 1. of this**
11 **paragraph shall be sent to the Commonwealth, the respondent's**
12 **attorney of record, the respondent's guardian ad litem, and all other**
13 **parties of record.**

14 (6) If the court determines that insufficient evidence has been presented to support a
15 finding that the respondent is **responsible for**~~[guilty of]~~ the charged crime against
16 him or her, the court shall order the immediate release of the respondent.

17 (7) ~~**Any**~~~~[No]~~ evidence or statement submitted by the respondent at the evidentiary
18 hearing shall **not** be admissible in any criminal prosecution or civil litigation.

19 ➔Section 29. KRS 202C.040 is amended to read as follows:

20 (1) A commitment hearing shall be held within **forty-five (45)**~~[twenty (20)]~~ days,
21 **unless the court orders a later hearing for good cause shown**, excluding weekends
22 and holidays, after the court finds that the evidence presented in an evidentiary
23 hearing pursuant to KRS 202C.030 supports a finding that the respondent is
24 **responsible for**~~[guilty of]~~ the charged crime against him or her by a preponderance
25 of the evidence.

26 (2) The commitment hearing may be conducted in an informal manner, consistent with
27 orderly procedures, and in a physical setting not likely to have a harmful effect on

1 the mental or physical health of the respondent. The hearing may be held by the
2 court in chambers, at a forensic psychiatric facility, or other suitable place.

3 (3) (a) The Commonwealth's attorney's office serving the county of criminal
4 prosecution which led to the finding that the respondent was incompetent to
5 stand trial shall present evidence regarding whether the respondent meets the
6 criteria for involuntary commitment under KRS 202C.050.

7 (b) The respondent ~~[and the respondent's guardian ad litem]~~ shall be afforded an
8 opportunity to testify, and the respondent's counsel shall conduct the
9 hearing on the respondent's behalf to present evidence~~[,]~~ and to cross-
10 examine any witnesses.

11 (c) The respondent's guardian ad litem shall participate in the proceeding in a
12 best-interest, friend-of-the-court capacity and may submit independent
13 recommendations to the court or jury, if a jury has been requested,
14 regarding the respondent's best interest.

15 (4) The manner of proceeding and the rules of evidence shall be the same as those in
16 any criminal proceeding. The standard of proof shall be proof beyond a reasonable
17 doubt. Proceedings shall be heard by the judge unless a party or the guardian ad
18 litem requests a jury.

19 (5) The respondent's right to the commitment hearing shall not be waived.

20 ➔Section 30. KRS 202C.050 is amended to read as follows:

21 (1) ~~A[No]~~ respondent shall not be involuntarily committed under this chapter unless
22 there is a determination that:

23 (a) The respondent presents, or would present if released, a danger to self or
24 others as a result of his or her mental condition; and

25 (b) ~~[The respondent needs care, training, or treatment in order to mitigate or~~
26 ~~prevent substantial physical harm to self or others;~~

27 ~~(c) The respondent has a demonstrated history or recent manifestation of criminal~~

1 ~~behavior that has endangered or caused injury to others or has a substantial~~
2 ~~history of involuntary hospitalizations under KRS Chapter 202A or 202B~~
3 ~~prior to the commission of the charged crime; or~~

4 ~~(d) —~~ A **presently available** less restrictive alternative mode of treatment would
5 endanger the safety of the respondent or others.

6 (2) When a respondent is involuntarily committed under this chapter, the cabinet shall
7 place that respondent in a forensic psychiatric facility designated by the secretary.

8 ➔ Section 31. KRS 202C.060 is amended to read as follows:

9 (1) (a) A review hearing to determine if a respondent involuntarily committed under
10 this chapter should remain in a forensic psychiatric facility shall be conducted
11 by the court that issued the initial order **in accordance with**~~[according to the~~
12 ~~provisions of]~~ subsection (2) of this section; and

13 (b) If at any point during the respondent's placement at a forensic psychiatric
14 facility it appears that the respondent no longer meets the criteria for
15 involuntary commitment under KRS 202C.050 because there has been a
16 material change in circumstances or there is new evidence to present, the
17 respondent or the respondent's guardian ad litem may request a review hearing
18 pursuant to this section.

19 (2) The schedule for review hearings shall be as follows:

20 (a) From the initial order of commitment, a ~~[standard]~~ review hearing shall be
21 conducted **no earlier**~~[not sooner]~~ than ninety (90) days and **no**~~[not]~~ later than
22 one hundred twenty (120) days; **and**

23 (b) **After the review hearing under paragraph (a) of this subsection, review**
24 **hearings shall be conducted not less than once every two (2) years unless a**
25 **review hearing has been requested under subsection (1)(b) of this**
26 **section**~~[For the first two (2) years after the initial order of commitment,~~
27 ~~standard review hearings shall be conducted not less than one hundred eighty~~

1 ~~(180) days and not more than two hundred ten (210) days from the most~~
2 ~~recent review;~~

3 ~~(c) Beginning two (2) years after the initial order of commitment, a standard~~
4 ~~review hearing shall be conducted not more than three hundred sixty-five~~
5 ~~(365) days from the most recent review hearing; and~~

6 ~~(d) A heightened review hearing shall be conducted not more than five (5) years~~
7 ~~from the initial order of commitment and, thereafter, not more than five (5)~~
8 ~~years from the most recent heightened review hearing].~~

9 (3) (a) Prior to each ~~[standard]~~ review hearing, the court shall cause the respondent to
10 be examined without unnecessary delay by two (2) qualified mental health
11 professionals, at least one (1) of whom is a physician. The qualified mental
12 health professionals shall, **no fewer than**~~[within]~~ seven (7) days prior to the
13 hearing, excluding weekends and holidays, certify to the court their findings
14 as to whether the respondent meets the criteria for involuntarily commitment
15 under KRS 202C.050.

16 (b) 1. In addition to the examinations required under paragraph (a) of this
17 subsection, the respondent shall undergo evaluations of competency at
18 least once every two (2) years to be conducted at a forensic psychiatric
19 facility.

20 2. Upon a finding by the forensic psychiatric facility that the
21 respondent's competency has been restored, the forensic psychiatric
22 facility shall provide written notice of restoration to the court, the
23 Commonwealth, the respondent's attorney of record, the respondent's
24 guardian ad litem, and all other parties of record within ten (10) days
25 of the determination of restoration.

26 3. Upon receipt of the notice of restoration of the respondent's
27 competency, a status conference shall be held within thirty (30) days,

1 **unless the court orders a later hearing for good cause shown.**

2 (4) A ~~{standard}~~ review hearing may be conducted in an informal manner, consistent
3 with orderly procedures, and in a physical setting not likely to have a harmful effect
4 on the mental or physical health of the respondent. The hearing may be held by the
5 court in chambers, ~~{or}~~ remotely from a forensic psychiatric facility, or **in**
6 **another**~~{other}~~ suitable place. The respondent shall be present in person or remotely
7 for all review hearings, unless presence is waived by the respondent through
8 counsel.

9 (5) **(a)** The Commonwealth's attorney's office serving the county of criminal
10 prosecution which led to finding that the respondent was incompetent to stand
11 trial shall present evidence regarding whether the respondent~~{remains~~
12 ~~incompetent to stand trial and}~~ continues to meet the criteria for involuntary
13 commitment under KRS 202C.050.

14 **(b)** The respondent, **through counsel,** ~~{and the respondent's guardian ad litem~~
15 ~~}~~ shall be afforded an opportunity to present evidence, and to cross-examine
16 any witnesses.

17 **(c) The respondent's guardian ad litem shall be permitted to participate in the**
18 **review hearing in a best-interest, friend-of-the-court capacity and may**
19 **submit independent recommendations to the court regarding the**
20 **respondent's best interest.**

21 (6) The manner of proceeding and the rules of evidence shall be the same as those in
22 any criminal proceeding. The standard of proof shall be proof beyond a reasonable
23 doubt. Proceedings shall be heard by a judge without a jury~~{, except that a~~
24 ~~respondent shall be entitled to a jury upon request if the respondent has not had a~~
25 ~~review hearing with a jury during the preceding twelve (12) months}.~~

26 (7) The respondent's right to this hearing shall not be waived.

27 (8) At the conclusion of a ~~{standard}~~ review hearing, the court shall make written

findings of fact concerning whether the criteria for involuntary commitment under KRS 202C.050 continue to be satisfied based upon proof beyond a reasonable doubt. If the court finds that the criteria continue to be satisfied, the court shall enter an order authorizing the continued care and treatment of the respondent at the forensic psychiatric facility. Otherwise, the court shall enter an order requiring the respondent to be discharged.}

~~(9) During a heightened review hearing, the procedures of a standard review hearing shall apply. Additionally, the qualified mental health professionals who evaluated the respondent in preparation for the hearing shall be required to give live testimony and answer questions before the court. The respondent shall be physically present in the courtroom for the hearing. If the respondent is unable to attend for any reason, the hearing shall be rescheduled to a time, place, and manner in which the respondent is able to attend.}~~

➔Section 32. KRS 202C.130 is amended to read as follows:

(1) Forensic psychiatric facilities ordered to receive an involuntarily committed respondent shall have standing to petition the Circuit Court for any necessary clarification or modification of orders or judgments entered in proceedings under this chapter and to appeal from final judgments or orders entered in proceedings which have not complied with the provisions of this chapter.

(2) A copy **of any motions filed under subsection (1) of this section** shall be sent to the involuntarily committed respondent, the respondent's guardian ad litem, ~~and~~ the respondent's attorney of record, **the Commonwealth, and all other parties of record**~~[of whatever pleadings are filed by the hospital]~~.

➔Section 33. KRS 202A.014 is amended to read as follows:

All proceedings for the involuntary hospitalization of **individuals with a mental illness**~~[mentally ill persons]~~ shall be initiated in the District Court of the county where the person to be hospitalized resides or in which he may be at the time of the filing of a

1 petition.

2 ➔Section 34. KRS 202A.026 is amended to read as follows:

3 ~~A[No]~~ person shall **not** be involuntarily hospitalized unless ~~the[such]~~ person is **an**
4 **individual with a mental illness**~~[a mentally ill person]:~~

- 5 (1) Who presents a danger or threat of danger to self, family or others as a result of the
6 mental illness;
- 7 (2) Who can reasonably benefit from treatment; and
- 8 (3) For whom hospitalization is the least restrictive alternative mode of treatment
9 presently available.

10 ➔Section 35. KRS 202A.231 is amended to read as follows:

- 11 (1) Upon receipt of a certificate of the United States Public Health Service or ~~[such]~~
12 ~~other~~ agency of the United States government that facilities are available for the
13 care or treatment of any person ~~[heretofore]~~ hospitalized in any mental hospital or
14 other institution in this state for the care of **individuals with a mental**
15 **illness**~~[mentally ill persons]~~ and that such person is eligible for such care or
16 treatment, the secretary, upon recommendation by any such hospital or institution in
17 this state, is ~~[hereby]~~ authorized to cause the transfer of any such person to the
18 United States Public Health Service or other agency of the United States
19 government for care or treatment. Upon effecting any such transfer, the
20 hospitalizing court shall be notified ~~[thereof]~~ by the secretary.
- 21 (2) Any person transferred as provided in this section shall be deemed to be placed in
22 the custody of the United States Public Health Service or other agency of the United
23 States government pursuant to the original hospitalization the same as if he **or she**
24 had been originally so hospitalized.
- 25 (3) ~~A[No]~~ person shall **not** be transferred to any agency of the United States if he **or**
26 **she** be confined pursuant to conviction of any felony or misdemeanor or if he **or**
27 **she** has been acquitted of the charge solely on the ground of mental illness unless

1 prior to transfer the court issuing the confining order shall enter an order to transfer
2 after the motion and hearing. Any person transferred as provided in this section to
3 any agency of the United States shall be hospitalized by such agency pursuant to the
4 original order of hospitalization.

5 ➔Section 36. KRS 202A.261 is amended to read as follows:

6 No public or private hospital, other than a state-operated or contracted mental hospital or
7 institution, shall be required to provide services under KRS 202A.008, 202A.011,
8 202A.028, 202A.041, 202A.051, 202A.071, ~~{202A.081,}~~202A.0811 to 202A.0831,
9 202A.101, 202A.141, 202A.241, 202A.251, 202A.261, 202A.271, 202B.170, 202B.200,
10 387.540, 504.085, 600.020, 645.020, 645.120, and 645.280 unless the hospital agrees to
11 provide the services. Any hospital shall make every reasonable attempt to cooperate with
12 the implementation of KRS 202A.008, 202A.011, 202A.028, 202A.041, 202A.051,
13 202A.071, ~~{202A.081,}~~202A.101, 202A.141, 202A.241, 202A.251, 202A.261,
14 202A.271, 202B.170, 202B.200, 387.540, 504.085, 600.020, 645.020, 645.120, and
15 645.280.

16 ➔Section 37. KRS 202A.271 is amended to read as follows:

17 Each public or private hospital, other than a state-operated or contracted mental hospital
18 or institution, which provides services under KRS 202A.008, 202A.011, 202A.028,
19 202A.041, 202A.051, 202A.071, ~~{202A.081,}~~202A.0811 to 202A.0831, 202A.101,
20 202A.141, 202A.241, 202A.251, 202A.261, 202A.271, 202B.170, 202B.200, 387.540,
21 504.085, 600.020, 645.020, 645.120, and 645.280 shall be paid for the services at the
22 same rates the hospital negotiates with the Department for Behavioral Health,
23 Developmental and Intellectual Disabilities or the regional community program for
24 mental health and for individuals with an intellectual disability.

25 ➔Section 38. KRS 387.540 is amended to read as follows:

26 (1) (a) Prior to a hearing on a petition for a determination of partial disability or
27 disability and the appointment of a limited guardian, guardian, limited

1 conservator, or conservator, an interdisciplinary evaluation report shall be
2 filed with the court. The report may be filed as a single and joint report of the
3 interdisciplinary evaluation team, or it may otherwise be constituted by the
4 separate reports filed by each individual of the team.

5 (b) If the court and all parties to the proceeding and their attorneys agree to the
6 admissibility of the report or reports, the report or reports shall be admitted
7 into evidence and shall be considered by the court or the jury if one is
8 impaneled.

9 (c) The report shall be compiled by at least three (3) individuals, including:

- 10 1. A physician, an advanced practice registered nurse, or a physician
11 assistant;
- 12 2. A psychologist licensed or certified under the provisions of KRS
13 Chapter 319; and
- 14 3. A person licensed or certified as a social worker or an employee of the
15 Cabinet for Health and Family Services who has at least one (1) year of
16 investigative experience and has completed training in conducting
17 decisional capacity assessments. The social worker shall, when possible,
18 be chosen from among employees of the Cabinet for Health and Family
19 Services residing or working in the area, and there shall be no additional
20 compensation for their service on the interdisciplinary evaluation team.

21 (2) At least one (1) person participating in the compilation of the report shall have
22 knowledge of the particular disability which the respondent is alleged to have or
23 knowledge of the skills required of the respondent to care for himself and his estate.

24 (3) If the respondent is alleged to be partially disabled or disabled due to mental illness,
25 at least one (1) person participating in the compilation of the interdisciplinary
26 evaluation report shall be a qualified mental health professional as defined in KRS
27 202A.011(13). If the respondent is alleged to be partially disabled or disabled due

1 to an intellectual disability, at least one (1) person participating in the compilation
2 of the evaluation report shall be a qualified professional in the area of intellectual
3 disabilities as defined in KRS 202B.010(12).

4 (4) The interdisciplinary evaluation report shall contain:

- 5 (a) A description of the nature and extent of the respondent's disabilities, if any;
- 6 (b) Current evaluations of the respondent's social, intellectual, physical, and
7 educational condition, adaptive behavior, and social skills. Such evaluations
8 may be based on prior evaluations not more than three (3) months old, except
9 that evaluations of the respondent's intellectual condition may be based on
10 individual intelligence test scores not more than one (1) year old;
- 11 (c) An opinion as to whether guardianship or conservatorship is needed, the type
12 of guardianship or conservatorship needed, if any, and the reasons therefor;
- 13 (d) An opinion as to the length of time guardianship or conservatorship will be
14 needed by the respondent, if at all, and the reasons therefor;
- 15 (e) If limited guardianship or conservatorship is recommended, a further
16 recommendation as to the scope of the guardianship or conservatorship,
17 specifying particularly the rights to be limited and the corresponding powers
18 and duties of the limited guardian or limited conservator;
- 19 (f) A description of the social, educational, medical, and rehabilitative services
20 currently being utilized by the respondent, if any;
- 21 (g) A determination whether alternatives to guardianship or conservatorship are
22 available;
- 23 (h) A recommendation as to the most appropriate treatment or rehabilitation plan
24 and living arrangement for the respondent and the reasons therefor;
- 25 (i) A listing of all medications the respondent is receiving, the dosage, and a
26 description of the impact of the medication upon the respondent's mental and
27 physical condition and behavior;

- 1 (j) An opinion whether attending a hearing on a petition filed under KRS
2 387.530 would subject the respondent to serious risk of harm;
- 3 (k) The names and addresses of all individuals who examined or interviewed the
4 respondent or otherwise participated in the evaluation; and
- 5 (l) Any dissenting opinions or other comments by the evaluators.
- 6 (5) The evaluation report may be compiled by a community center for mental health or
7 individuals with an intellectual disability, a licensed facility for mentally ill or
8 developmentally disabled persons, if the respondent is a resident of such facility, or
9 a similar agency.
- 10 (6) In all cases where the respondent is a resident of a licensed facility for mentally ill
11 or developmentally disabled persons and the petition is filed by an employee of that
12 facility, the petition shall be accompanied by an interdisciplinary evaluation report
13 prepared by the facility.
- 14 (7) Except as provided in subsection (6) of this section, the court shall order
15 appropriate evaluations to be performed by qualified persons or a qualified agency.
16 The report shall be prepared and filed with the court and copies mailed to the
17 attorneys for both parties at least ten (10) days prior to the hearing. All items
18 specified in subsection (4) of this section shall be included in the report.
- 19 (8) If the person evaluated is a poor person as defined in KRS 453.190, the examiners
20 shall be paid by the county in which the petition is filed upon an order of allowance
21 entered by the court. Payment shall be in an amount which is reasonable as
22 determined by the court, except no payment shall be required of the county for an
23 evaluation performed by a salaried employee of a state agency for an evaluation
24 performed within the course of his employment. Additionally, no payment shall be
25 required of the county for an evaluation performed by a salaried employee of a
26 community center for mental health or individuals with an intellectual disability or
27 private facility or agency where the costs incurred by the center, facility, or agency

1 are reimbursable through third-party payors. Affidavits or other competent evidence
2 shall be admissible to prove the services rendered but not to prove their value.

3 (9) The respondent may file a response to the evaluation report no later than five (5)
4 days prior to the hearing.

5 (10) The respondent may secure an independent evaluation. If the respondent is unable
6 to pay for the evaluation, compensation for the independent evaluation may be paid
7 by the county in an amount which is reasonable as determined by the court.

8 ➔Section 39. KRS 625.090 is amended to read as follows:

9 (1) The Circuit Court may involuntarily terminate all parental rights of a parent of a
10 named child, if the Circuit Court finds from the record by clear and convincing
11 evidence that:

- 12 (a) 1. The child has been adjudged to be an abused or neglected child, as
13 defined in KRS 600.020~~[(1)]~~, by a court of competent jurisdiction;
- 14 2. The child is found to be an abused or neglected child, as defined in KRS
15 600.020~~[(1)]~~, by the Circuit Court in this proceeding;
- 16 3. The child is found to have been diagnosed with neonatal abstinence
17 syndrome at the time of birth, unless his or her birth mother:
- 18 a. Was prescribed and properly using medication for a legitimate
19 medical condition as directed by a health care practitioner that may
20 have led to the neonatal abstinence syndrome;
- 21 b. Is currently, or within ninety (90) days after the birth, enrolled in
22 and maintaining substantial compliance with both a substance
23 abuse treatment or recovery program and a regimen of prenatal
24 care or postnatal care as recommended by her health care
25 practitioner throughout the remaining term of her pregnancy or the
26 appropriate time after her pregnancy; or
- 27 c. In the absence of a prescription for the treatment of a legitimate

1 medical condition, agrees, prior to discharge from the hospital, to
2 participate in a court-ordered assessment by a drug treatment
3 provider and the assigning of a certified peer support specialist for
4 referral to appropriate treatment, and agrees to participate in
5 treatment which shall commence within ninety (90) days after the
6 birth; or

7 4. The parent has been convicted of a criminal charge relating to the
8 physical or sexual abuse or neglect of any child and that physical or
9 sexual abuse, neglect, or emotional injury to the child named in the
10 present termination action is likely to occur if the parental rights are not
11 terminated;

12 (b) 1. The Cabinet for Health and Family Services has filed a petition with the
13 court pursuant to KRS 620.180 or 625.050; or

14 2. A child-placing agency licensed by the cabinet, any county or
15 Commonwealth's attorney, or a parent has filed a petition with the court
16 under KRS 625.050; and

17 (c) Termination would be in the best interest of the child.

18 (2) ~~A[No]~~ termination of parental rights shall **not** be ordered unless the Circuit Court
19 also finds by clear and convincing evidence the existence of one (1) or more of the
20 following grounds:

21 (a) That the parent has abandoned the child for a period of not less than ninety
22 (90) days;

23 (b) That the parent has inflicted or allowed to be inflicted upon the child, by other
24 than accidental means, serious physical injury;

25 (c) That the parent has continuously or repeatedly inflicted or allowed to be
26 inflicted upon the child, by other than accidental means, physical injury or
27 emotional harm;

- 1 (d) That the parent has been convicted of a felony that involved the infliction of
2 serious physical injury to any child;
- 3 (e) That the parent, for a period of not less than six (6) months, has continuously
4 or repeatedly failed or refused to provide or has been substantially incapable
5 of providing essential parental care and protection for the child and that there
6 is no reasonable expectation of improvement in parental care and protection,
7 considering the age of the child;
- 8 (f) That the parent has caused or allowed the child to be sexually abused or
9 exploited;
- 10 (g) That the parent, for reasons other than poverty alone, has continuously or
11 repeatedly failed to provide or is incapable of providing essential food,
12 clothing, shelter, medical care, or education reasonably necessary and
13 available for the child's well-being and that there is no reasonable expectation
14 of significant improvement in the parent's conduct in the immediately
15 foreseeable future, considering the age of the child;
- 16 (h) That:
- 17 1. The parent's parental rights to another child have been involuntarily
18 terminated;
- 19 2. The child named in the present termination action was born subsequent
20 to or during the pendency of the previous termination; and
- 21 3. The conditions or factors which were the basis for the previous
22 termination finding have not been corrected;
- 23 (i) That the parent has been convicted in a criminal proceeding of having caused
24 or contributed to the death of another child as a result of physical or sexual
25 abuse or neglect;
- 26 (j) That the child has been in foster care under the responsibility of the cabinet
27 for fifteen (15) cumulative months out of forty-eight (48) months preceding

1 the filing of the petition to terminate parental rights; or

2 (k) That the child has been removed from the biological or legal parents more
3 than two (2) times in a twenty-four (24) month period by the cabinet or a
4 court.

5 (3) In determining the best interest of the child and the existence of a ground for
6 termination, the Circuit Court shall consider the following factors:

7 (a) Mental illness as defined by KRS 202A.011~~[(9)]~~, or an intellectual disability
8 as defined by KRS 202B.010(9) of the parent as certified by a qualified
9 mental health professional, or a disability as defined in KRS 199.011, if the
10 mental illness, intellectual disability, or disability renders the parent
11 consistently unable to care for the immediate and ongoing physical or
12 psychological needs of the child for extended periods of time;

13 (b) Acts of abuse or neglect as defined in KRS 600.020~~[(4)]~~ toward any child in
14 the family;

15 (c) If the child has been placed with the cabinet, whether the cabinet has, prior to
16 the filing of the petition:

17 1. Made reasonable efforts as defined in KRS 620.020 to reunite the child
18 with the parents unless one or more of the circumstances enumerated in
19 KRS 610.127 for not requiring reasonable efforts have been
20 substantiated in a written finding by the District Court; or

21 2. Provided a parent with a disability as defined in KRS 199.011 with
22 targeted adaptive and supportive services based on an individual
23 assessment of the parent, or has received a written acknowledgement
24 from the parent knowingly and affirmatively rejecting the offered
25 services;

26 (d) The efforts and adjustments the parent has made in his or her circumstances,
27 conduct, or conditions to make it in the child's best interest to return the child

1 to his or her home within a reasonable period of time, considering the age of
2 the child;

3 (e) The physical, emotional, and mental health of the child and the prospects for
4 the improvement of the child's welfare if termination is ordered; and

5 (f) The payment or the failure to pay a reasonable portion of substitute physical
6 care and maintenance if financially able to do so.

7 (4) If the child has been placed with the cabinet, the parent may present testimony
8 concerning the reunification, adaptive or supportive services offered by the cabinet,
9 and whether additional services would be likely to bring about lasting parental
10 adjustment enabling a return of the child to the parent.

11 (5) If the parent proves by a preponderance of the evidence that the child will not
12 continue to be an abused or neglected child as defined in KRS 600.020~~[(1)]~~ if
13 returned to the parent, or if the parent proves by a preponderance of the evidence
14 that appropriate and specifically targeted adaptive or supportive services based
15 upon an individual assessment of the parent have not been offered or provided to
16 the parent, the court in its discretion may determine not to terminate parental rights.

17 (6) Upon the conclusion of proof and argument of counsel, the Circuit Court shall enter
18 findings of fact, conclusions of law, and a decision as to each parent-respondent
19 within thirty (30) days either:

20 (a) Terminating the right of the parent; or

21 (b) Dismissing the petition and stating whether the child shall be returned to the
22 parent or shall remain in the custody of the state.

23 ➔Section 40. KRS 202A.006 is amended to read as follows:

24 This chapter may be cited as the ~~[""]Kentucky Mental Health Hospitalization~~
25 Act.~~[""]~~

26 ➔Section 41. The following KRS section is repealed:

27 202A.081 Court-ordered community-based outpatient treatment.

1 ➔Section 42. By November 1, 2026, the Cabinet for Health and Family Services
2 shall provide a report to the Legislative Research Commission for referral to the Interim
3 Joint Committee on Health Services and the Interim Joint Committee on Judiciary
4 describing the existing services, treatments, and supports for mental illness and serious
5 mental illness available to persons who are subject to proceedings under KRS Chapter
6 202A and 202C and making recommendations for ways to strengthen, increase, and
7 broaden these services, treatments, and supports as appropriate. The descriptions shall
8 include type of providers, eligibility criteria, accessibility, and geographical locations of
9 the services, treatments, and supports.

10 ➔Section 43. The Cabinet for Health and Family Services and Department for
11 Behavioral Health, Developmental and Intellectual Disabilities shall engage the services
12 of the Treatment Advocacy Center to develop and implement a statewide training
13 program to facilitate implementation of Sections 8 to 41 of this Act and ensure consistent
14 application of Sections 8 to 41 of this Act across state and local agencies.

15 ➔Section 44. Section 1 of this Act may be cited as the Family Preservation and
16 Accountability Act.

17 ➔Section 45. Section 4 of this Act takes effect January 1, 2027.

18 ➔Section 46. Sections 8 to 41 and 43 of this Act take effect October 1, 2026.